The Response of Camden PCT King's Cross HIA Project to LBC and LBI on the Argent St George Planning Applications for King's Cross Central

Part I:
Potential Impacts on Health and Well-Being of the Redevelopment of King's Cross Central

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References

I) Reference summary of ways to realise the potential benefits of the King's Cross Central redevelopment and key issues that need to be addressed

Employment opportunities

Realising the potential benefits

We endorse Argent St George's support for:

- construction training projects;
  an employer-led Local Employment Strategy

We support Argent St George in their intention:

- to give early support to employment brokerage and skills training initiatives that match identified employers' needs with skills shortages or residents' aspirations;
- to encourage occupiers to engage with local employment initiatives in the manner most appropriate to them.

However, we ask that:

- Argent St George commit themselves to a King's Cross Education, Employment and Training Strategy as outlined in Section 2.8.3 of the Joint Planning and Development Brief
- Argent St George explore the issue of employing local people when negotiating with potential occupiers of King's Cross Central;
- any employment brokerage schemes for local people coincide with the phasing of development zones on the Main site, such that local people can try for the employment opportunities as they are arise;
- recruitment by all business occupiers at King's Cross Central is monitored to ascertain whether local people are gaining jobs, and if not, why not;
- Argent St George address the content of section 2.8.3 in the Joint Planning and Development Brief and strengthen the Code of Construction Practice (COCP) to include measures for local recruitment - as it stands, COCP only mentions advertising jobs locally
- any training and skills development programmes for local people need to address barriers to course uptake, e.g. by providing access to crèche or day care facilities, by using local venues as training locations, ensuring the affordability of any training schemes, and publicising the courses effectively, i.e. information available at points in the community not just at Job Centres;
- effective training courses in English language skills need to be developed and targeted at people in the black and minority ethnic communities, e.g. Bangladeshi, Somali, and Turkish speakers, to address the deficit in English language skills, which may increase their chances of obtaining employment, of obtaining better-quality employment or of being able to attend a further training course to improve their employment prospects.

Key issues to be addressed

- How many of the job opportunities identified by Argent St George will be lost through the relocation of companies?
- Argent St George need to provide evidence to support their assumptions that 5% of residents in their Central Impact Zone and 25% of residents in their Wider Impact Zone will be able to take advantage of the employment opportunities at King's Cross Central.
- What interventions would be necessary to achieve 40% employment of local residents at King's Cross Central (as claimed by Argent St George in their Regeneration Strategy and in the Environmental Statement)?

- How many of the job opportunities on offer at King's Cross Central are likely to go to the long-term unemployed in the area?

Housing

Realising the potential benefits

We support:

- London Borough of Camden's Unitary Development Plan Policy KC4 of 50% affordable housing for the first additional 1000 residential units to be built on the Main site at King's Cross Central – this would generate 500 affordable homes;
- London Borough of Camden's targets of 35% social housing for renting and of 15% for essential workers and other intermediate occupiers for the first additional 1000 residential units to be built on the Main site at King's Cross Central – this would generate 350 homes for social renting and 150 for intermediate occupiers. For units over the first additional 1000 units on the main site, 50% affordable housing comprising a significant element of social housing for renting and housing for essential workers (may also include other forms of low cost housing);
- London Borough of Islington's policy in the Unitary Development Plan of up to 35% of affordable housing on the Triangle site, but would urge for the maximum of 35% to be negotiated – if so, this would generate up to 87 units of affordable housing.

Key issues to be addressed

- The amount of social housing for renting to be built on the Main site.
- Whether Argent St George's intend to build housing at King's Cross Central as a mixed tenure development – there is the suggestion that affordable housing will be in separate blocks to market housing with respect to the Triangle site.
- Whether Argent St George intend to build sheltered and residential accommodation at King's Cross Central (as laid out in the Joint Planning and Development Brief).
- Whether Argent St George's intention to build a relatively low proportion of family accommodation at King's Cross Central is acceptable given the housing needs in Camden.
- The amount of housing, and the proportion of affordable housing, that Argent St George intend to build on the Triangle site.

Access to healthcare facilities and services

Realising the potential benefits

- It is vital that new healthcare facilities are provided on site to meet the demands of new residents, workers, visitors/tourists, and passengers, especially as some local services are overstretched at present and increased demand could compromise the volume and quality of services provided to already existing communities.
- The Estates and Facilities Team for Camden and Islington Primary Care Trusts will be submitting a detailed proposal for healthcare facilities on the King's Cross Central site as part of the section 106 negotiations between Argent St George and London Borough of Camden, and London Borough of Islington, including specifications about location and accessibility both for service users and emergency vehicles.

Key issues to be addressed

- Whether Argent St George intend the walk-in centre that could be built at King's Cross Central to be run by the public or the private sector.
- Whether the triangle site which according to Argent St George documents includes a 'medi-centre' will be redeveloped, and if not where Argent St George intend to locate healthcare facilities on the main site

Access to affordable nutritious food

Realising the potential benefits

We suggest that:

- at least one of the options for a supermarket at King's Cross Central is realised, and Argent St George negotiate with a chain that produces good-quality but affordable food (e.g. Asda, Costcutter, Lidl);
- whichever supermarket chain is recruited to the site, explore whether they are prepared to be involved in Healthy Eating Campaigns targeted at the vulnerable in the community within the framework of the PCTs' Healthy Eating Policies and Implementation Plans, especially given the rates of coronary heart disease, and cancers in the area;
- the shopping facility is placed in an accessible location on site.

Key issues to be addressed

- That Argent St George ensure at least one shopping facility/supermarket at King's Cross Central supplies good-quality nutritious food at affordable prices
- Whether the triangle site which according to Argent St George documents includes a relatively small supermarket/shopping facility will be redeveloped, and if not where Argent St George intend to locate this outlet on the main site.

Access to education, community and cultural facilities

Realising the potential benefits

We strongly suggest:

- that community facilities are provided on site in tandem with the phasing of the construction of residential units;

We suggest that:

- smaller neighbourhood meeting places or centres could be built in the some of the development zones containing residential units;
- new residents on site be given the opportunity to participate in the growth and development of any community facilities;
- consideration is given to the need for facilities that can be used as faith facilities, especially in view of the lack of such facilities for the Muslim communities community in the area;
- the needs of children, young people, and older people are taken into account when planning community facilities;
- the cultural needs of the various black and minority ethnic groups in the area are taken into account when planning community facilities.

Key issues to be addressed

- If negotiations with the Higher Education Institution to occupy 40,000 square metres in Development Zone L are not successful, will Argent St George allocate this floor space to another land use, i.e. it will not be used for educational, community or cultural uses?
- Whether Argent St George intend to build only one dedicated community centre/space at King's Cross Central – the only definite mention of such a facility is for the Triangle site, there is no definite mention of one for the Main site.
- Whether the triangle site which according to Argent St George documents includes a dedicated space for the community and childcare facilities (crèche) will be redeveloped, and if not where Argent St George intend to locate these facilities on the main site.

Access to leisure facilities

Realising the potential benefits

We suggest:

- that the balance of assembly and leisure uses on site is built to be socially inclusive, such that all residents in the area, whether new or existing, could access them;
- that, where relevant, leisure facilities, but especially sports facilities, have the capacity to provide culturally sensitive services, e.g. swimming pool that is managed in such a way as to accommodate the needs of the Muslim communities in the area;
- that the entrance/usage cost of leisure facilities, including sports facilities, are affordable, especially in view of the number of people living on a low income in the area surrounding the site;
- the needs of the relatively young resident population surrounding the site are considered when deciding on the balance of leisure and assembly uses.
- that the estate management strategy/protocol takes into consideration the potential impacts of a 24 hour economy on residents and other users, and incorporates a mechanism to liaise with the licensing department at the Council.

We endorse:

- the inclusion of a leisure centre with sports facilities in Zone W, although it is important to ensure that these facilities are affordable;
- the inclusion of a specialist play area in Zone N, and other specialist play areas, for 1-8 year olds;
- the inclusion of places for 8-16 year olds to play;
- the inclusion of areas/facilities for 5-a-side football.

Key issues to be addressed

- The affordability of leisure services built at King's Cross Central
- The social inclusiveness of leisure services built at King's Cross Central
- The cultural inclusiveness of leisure services built at King's Cross Central
- How Argent St George intend to manage the potential for the development of a 24-hour economy at King's Cross Central, and the nuisance that this may cause to residents on site and in the surrounding areas
- Whether the triangle site which according to Argent St George documents includes a leisure facility (either a sports hall or a swimming pool) will be redeveloped, and if not where Argent St George intend to locate these facilities on the main site.

Access to transport

Realising the potential benefits

We endorse:

- the intention to introduce bus services to the King's Cross Central site, and to improve and enhance existing services;
- the plans to make active transport options viable alternatives to the use of non-active transport;
- the principle of developing and implementing a Green Travel Plan for the King's Cross Central site, and the decision to nominate a Travel Coordinator to work with occupiers and other stakeholders to deliver on the objectives.

We suggest:

- that the nominated Travel Coordinator liaise with healthcare providers in the area, particularly the primary care trusts, to ensure that any health-related initiatives to increase daily physical activity in relation to transport are coordinated, and opportunities for joint working are identified and capitalised upon.

Key issues to be addressed

- The need for further investigation of the impacts of the traffic generated by King's Cross Central on existing local communities, including parking by non-residents, road traffic to and from the railway termini, and the use of local streets off-site as short-cuts.

Access to green and open spaces

Realising the potential benefits

We endorse:

- the inclusion of the "Long Park" in the north of the King's Cross Central site;
- the inclusion of planting schemes throughout the site as illustrated in the Landscape proposals.

We suggest:

- that the Estate Management strategy/protocol covers the Long Park appropriately in that green spaces can be a location for drug abuse and conduct offences, including various types of aggression.

We ask Argent St George to consider:

- identifying further possibilities to introduce green space into the King's Cross Central redevelopment.

Quality of landscape/Level of biodiversity

Realising the potential benefits

- We endorse Argent St George's development of a wildlife habitat on the Triangle site, especially as it is designed to house black redstarts, a rare breeding bird in the UK. This would increase the overall amount of wildlife habitat in the area, and owing to its relative proximity to Camley Street Natural Park may act to support the existing wildlife populations in the area.

Quality of townscape

Realising the potential benefits

We welcome:

- Argent St George's intention behind the preparation of the Urban Design Statement.

We endorse:

- Argent St George's consideration of the potential to incorporate green and brown roofs on the Main site and the Triangle site, which will also have a positive effect on microclimate at King's Cross Central.

We ask:

- Argent St George to consider effective and appropriate ways of promoting social inclusion within King's Cross Central, including the design of good-quality open-access facilities

Crime and community safety

Realising the potential benefits

We support:

- the use of crime prevention design principles (secure by design) in the design of the King's Cross Central redevelopment;
- the introduction of an Estate Management Team to help deal with community safety issues not only on the King's Cross Central site but also around the margins of the site into the existing local communities – however, it is vital to address the issue of displacement of crime and antisocial behaviour, and what this could mean for other communities and service providers such as the police, and the street warden service

Key issues to be addressed

- The displacement of certain types of crime and antisocial behaviour, e.g. drug-related, to other parts of Camden and Islington, but in particular to the surrounding estates.
- The possibility that the pattern of crime and antisocial behaviour traditionally seen in the King's Cross area may change with the redevelopment of the area, and as a result of the operation of the Port/transport hub, for example, increase in arms smuggling or drugs smuggling, and an increase in the risk of a terrorist attack.
- The need to ensure that the concentration of office space in the southern part of King's Cross Central does not create a "no-go area" at night.

Public safety: incidents or accidents at or affecting King's Cross Central

Minimising the potential for harm

We ask that:

- Argent St George and landowners take responsibility for their contribution to Emergency Planning and Port Health as laid out in paragraph 2.10.10 of Joint Planning and Development Brief.

Key issues to be addressed

- Argent St George need to confirm what arrangements will be made to ensure that all occupiers at King's Cross Central have appropriate arrangements in place for the management of emergencies – there also needs to be liaison between the major transport operators at the Port/transport interchange.

Construction impacts

Minimising the potential for harm

We ask that:

- any 24-hour working is kept to an absolute minimum, especially in view of the findings of the Planning Inquiry into 24-hour working for the Channel Tunnel Rail Link (CTRL) Project.
- the communities/residents identified by Arup to be most affected by King's Cross Central construction works (people in Rufford Street, Gifford Street, York Way, new housing on corner of Rufford and Gifford Streets, and houseboats on Regent's Canal) are met with every two weeks and consulted about the works in the same way as residents in Coopers Lane and Goldington Estates following the Planning Inquiry into 24-hour working on the CTRL Project.
- Argent St George identify more effective ways of communicating with the local communities, particularly those from black and minority ethnic groups, than those identified in the COCP, which lists a focal point for information dissemination via a helpline, website, information boards and exhibitions.
- Argent St George consider including a behavioural Code of Conduct for nominated contractors. During the Community Workers in HIA Project, girls and young women in the local community have reported being subject to inappropriate comments by CTRL construction workers, which they have found offensive and intimidating.
- Argent St George consider setting up regular noise monitoring and instituting a mechanism to investigate and deal with noise complaints.

Key issues to be addressed

- The need for Argent St George to produce information and consultation documents for the community in the main languages of the Black and minority ethnic groups in the area.
- The need for Argent St George to develop more effective ways of communicating with existing communities around the site, especially those from Black and minority ethnic groups.

II) Purpose of the response of Camden PCT to LBC and LBI on the Planning Applications submitted by Argent St George for King's Cross Central

The overall aim of the King's Cross HIA Team in responding to the planning applications and supporting documents for King's Cross Central is:

> to support decision-makers at the Local Planning Authorities in their determination of the planning applications relating to King's Cross Central by providing them with information for consideration that highlights the implications of the redevelopment for people's lives and in particular their health and well-being

The objectives of the KX HIA Team in responding to the planning applications and supporting documents are:

- to provide an analysis of the likely content of the redevelopment at King's Cross Central (see Part II of the Response – Analysis of the Planning Applications and Supporting Documents for King's Cross Central);
- to identify what the content of the redevelopment will mean in terms of deliverables or outputs, and the major trade-offs this could present (see Part II of the Response – Analysis of the Planning Applications and Supporting Documents for King's Cross Central);
- to identify the contribution the redevelopment could make to reducing the inequalities gap in Camden (see Sections 2 and 3);
- to identify what the various deliverables or outputs will mean for people affected by the redevelopment, in terms of their economic, social and environmental well-being and the consequent impacts on their health (see Sections 2 and 3);
- to identify the issues decision-makers at the Local Planning Authorities need to consider from the perspective of health and well-being, and the ways in which any potential changes in health arising as a result of the redevelopment could be addressed (see Section 3).

In addition, in response to a request from the London Borough of Camden:

- to provide a critique of relevant chapters of the Environmental Statement, in particular Chapters 12 and 13 (see Part III of the Response – Commentary on Chapters 12 and 13 of Argent St George's Environmental Statement').

Section 1: Approach of King's Cross Health Impact Assessment Team

In Section 1, we outline the role of the King's Cross Health Impact Assessment (KX HIA) Team in relation to the work that has led up to the submission of the planning applications for King's Cross Central, and the approach the KX HIA Team has taken in response to the planning applications as submitted.

1.1 Health Impact Assessment of the Redevelopment of the King's Cross area

Since April 2002, Camden Primary Care Trust, with the support of Islington Primary Care Trust, has been leading a health impact assessment (HIA) of the redevelopment of the King's Cross area. The HIA covers six major redevelopments, including King's Cross Central.

Over the last two and a half years, the King's Cross HIA (KX HIA) Team has commented upon the following Argent St George (ASG) documents in relation to the redevelopment of King's Cross Central:

- two early drafts of the Health Chapter in the environmental impact assessment (EIA) being prepared in support of the planning applications for King's Cross Central;
- Argent St George's Third Consultation Document about King's Cross Central;
- the draft scope of the EIA prepared in support of the planning applications for King's Cross Central.

In addition, the KX HIA Team have commented on documents produced by the Local Planning Authorities (LPAs) – London Borough of Camden and London Borough of Islington – that are relevant to the redevelopment of King's Cross Central:

- the draft Joint Planning and Development Brief;
- the draft Local Context Assessment for the London Borough of Islington;
- stages in the evolution of the chapter relating to the King's Cross Opportunity Area (KXOA) in the Unitary Development Plan (UDP) for the London Borough of Camden.

1.2 Challenges presented by the planning applications for King's Cross Central

There are several challenges presented by the planning applications for King's Cross Central:

- the applicants have submitted planning applications that incorporate a degree of flexibility with respect to the amount of floor space allocated to each category of land use – thus, they have applied for a fixed total of floor space for both sites but within that "envelope" they have indicated only the maximum amount of floor space provided for in each category of land use (see Tables 1.1 and 1.2 in Part II of the Response) provided for in either individual development zones, or each grouping of development zones, on the Main site; therefore, it is not possible to be certain about the exact nature of the

development, what it will contain, and what it will deliver in terms of jobs, houses, and health, education, community, cultural and leisure facilities;

- in terms of the opportunities that the redevelopment of King's Cross Central might generate, such as employment opportunities and residential units built, Argent St George have presented these figures as totals that span the entire time period of construction and operation (see Tables 2.1-2.3 in Part II of the Response) – thus, it is not possible to identify what and how many opportunities will arise at which time-point in a period of 12 or more years;
- over half the development zones (A, B, J, K, L, M, P,Q, R, S, T, W) comprise more than one building or block (ranging from 2 to 7); however, the applicants do not give the floor space for each building or block within a zone, nor do they indicate when individual buildings or blocks will be started or completed – they give only a total amount of floor space for each zone and a start and completion date for each zone, therefore, it is not possible to identify the delivery dates for certain buildings within a zone nor the amount and use of floor space within them with any accuracy.

In addition, despite the willingness of the Local Planning Authorities (LPAs) to grant extensions, the consultation period for statutory agencies is short in relation to the number of documents (a total of 52) that need to be considered in order to compose a useful response.

These challenges mean that it has been difficult to predict the impacts of the redevelopment of King's Cross Central as outlined in the planning applications and supporting documents in the form in which they have been submitted.

1.3 The approach taken by the King's Cross HIA Team

Given the challenges presented by the planning applications (outlined above), the approach taken by the KX HIA Team has been as follows:

- to analyse the planning applications and identify what they will mean in terms of land uses and the associated outputs from those land uses, and the potential trade-offs involved;
- to assess the outputs from the land uses in terms of the implications over time for existing local communities, new residents, and the working and passenger populations, and the issues that may need to be addressed by decision-makers at the Local Planning Authorities;
- to identify what those implications might mean for the health and well-being of these groups of people and ways in which it will be advisable to address potential negative or positive changes in health.

Section 2: Overall effect of the redevelopment at King's Cross Central on inequalities and social exclusion

Although Camden is a wealthy borough, some of the people in Camden experience severe inequalities, and some of these inequalities are increasing.

In fact, Camden is the most polarised borough in London with the gap between the most and least deprived wards on the Index of Multiple Deprivation (IMD) being bigger than that in any other borough. These inequalities are also reflected in levels of health and well-being.

Thus, an over-riding policy driver for the borough is to reduce the gap between the most and least deprived, and to ensure that people who are deprived are not excluded from gaining work, and accessing the full range of services, all of which can have a positive effect on health. It is also a priority for the Primary Care Trust to reduce health inequalities.

These policy drivers on health and other inequalities are reflected in the operation of a Health Action Zone in Camden and Islington, and a regeneration area in Camden Central which bounds the western edge of the King's Cross Central site.[1]

Thus, the key questions to be considered in any HIA of the Planning Applications and Supporting Documents for the redevelopment of King's Cross Central are:

- "Will the redevelopment help to reduce the inequalities gap?"
- "Will the redevelopment reduce social exclusion?"
- "Will the redevelopment promote social inclusion?"
- "Will the redevelopment promote social integration and community cohesion?"

When asking these questions, it is important to be aware that, with redevelopment, the inequalities gap may appear to be reduced (or possibly eliminated) and social exclusion reduced but these effects are the result of a relatively healthy population being introduced into the area, e.g. residents in any new market housing who are likely to be affluent and healthy.

If there is to be a true reduction in inequalities, it is essential that the situation is improved for those experiencing the greatest deprivation and severest inequalities (access to employment, good-quality housing, good-quality services and facilities, etc) rather than for such vulnerable people to be displaced from the area into another where conditions are worse, known as gentrification. Improving the situation of people who are vulnerable by reason of deprivation and inequalities would promote social inclusion, and allow them the possibility of remaining in an area to benefit from redevelopment.[2]

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[1] Camden is also a member of the World Health Organization's Healthy Cities Network.

[2] When reducing the inequalities gap, it is important not to increase the level of deprivation or inequalities experienced by the least deprived.

In Section 2, we will explore this issue in terms of the overall impact of the redevelopment.

2.1 Does gentrification and displacement occur with redevelopment?

In the PCTs' response to Argent St George's third consultation document, we raised the possibility that gentrification and displacement could occur at King's Cross as a result of redevelopment affecting existing local communities around the site (see Box 2.1 for evidence base). Gentrification, i.e. the rehabilitation of working-class areas and derelict housing and the consequent transformation of an area into a middle-class neighbourhood (Smith and Williams, 1986), can lead to displacement as a result of factors such as:

- rising land values;
- new housing;
- changes to the employment sectors.

Box 2.1 Economic development, gentrification and displacement

What does the evidence base show?

- Economic development can be an important factor in developing social inclusion but neighbourhoods require local work opportunities in order to develop the bridging ties necessary to generate social capital and better health (Catell, 2001).
- Education, training and learning play important roles in providing the basis for economic growth (Organisation for Economic Co-operation and Development - OECD, 2001).
- In a longitudinal study of long-term non-migrating residents in Tennessee, it was found that they did not on average benefit economically from regeneration, and experienced a significant decrease in average household income (Glenn et al, 1998). In addition, residents had a statistically significant worsening of physical health status, which was tentatively attributed to the direct or indirect effects of a decline in family income. Local economic development can leave long-term area residents poorer and less healthy, an effect which may be masked by an increase in healthier, wealthier inward migrants.
- Neighbourhood 'improvements' may displace social problems rather than solve them, and this will not help to reduce inequalities in housing and in health (Curtis et al, 2002).
- Gentrification appears to improve the physical and social fabric of an area, however, social problems are usually evacuated through the 'improvement' of neighbourhoods: the subsequent absence of social problems is used as evidence that gentrification has positive social impacts (Atkinson, 1999).
- Displacement has social costs, such as increased housing need, overcrowding in 'hidden households', and homelessness (Atkinson, 1999).
- People who have been displaced are likely to feel resentment, disenfranchisement, and exclusion (Atkinson, 1999).
- Displacement also has neighbourhood effects, that is, the impairment of social networks, and a reduction in service provision (Atkinson, 1997). These neighbourhood effects will affect vulnerable groups adversely - they are less able to cope with the psychological and financial costs of displacement.

Argent St George have given the potential for gentrification and displacement consideration in both the Environmental Statement (Chapter 12) and the Regeneration Strategy (pages 44-45). In the Regeneration Strategy, they cite work by Atkinson at

the ESRC Centre for Neighbourhood Renewal that shows the positive and negative effects that accompany redevelopment (see Table 2.1).

Argent St George then cite further work of Atkinson's which shows that local circumstances determine whether positive or negative effects are seen with redevelopment. Thus, the key question to address is:

- "Will gentrification and displacement occur at King's Cross?".

Table 2.1: Positive and negative effects of redevelopment

Positive effects of redevelopment	*Negative effects of redevelopment*
▪ Physical renewal	▪ Displacement of residents
▪ Increased tax revenues	▪ Displacement of businesses
▪ Reduced crime	▪ Increased crime
▪ Improved local services	▪ Changes to local services
▪ Increased social mix	▪ Social displacement
▪ Increased population stability	▪ Community conflict

2.2 Will gentrification and displacement occur at King's Cross?

From the work done on the HIA, and in particular Argent St George's Third Consultation document, we know that the communities living around the King's Cross Central site are particularly vulnerable in terms of the health and other inequalities they experience (see Boxes 2.2 and 2.3).

Argent St George argue against displacement of vulnerable groups, pointing out that as 58% of households in the area surrounding the site are in affordable housing for social renting it is unlikely that these households will be displaced.

However, gentrification, in terms of the transformation of the King's Cross Central site into a middle-class neighbourhood, is inevitable, especially as:

- Argent St George argue for a high proportion of market housing on site (maximum of 70% of residential units) and, thus far, have not made a commitment to providing 50% affordable housing as laid out in London Borough of Camden's Unitary Development Plan (UDP);
- Argent St George argue for a "high" proportion of intermediate housing within the affordable housing component in any units over the first 1000 built on site and, thus far, have not made a commitment to providing 35% affordable housing for social renting as laid out in London Borough of Camden's UDP.

Despite this, Argent St George also argue that the physical form, social structure and landownership patterns in the area provide inherent protections against the negative effects of gentrification.

Thus, taking local conditions and circumstances into account, we have assessed the likelihood of which positive and negative effects of redevelopment will occur at King's Cross (see Table 2.2). As can be seen, although some of the potential positive effects are assessed as definite, others are not and for them to be realised they depend

on other factors. Moreover, the assessment shows that some of the potential negative effects are likely to occur, and may have a differential effect on the most vulnerable in the community.

Box 2.2 Key facts about the determinants of health in existing local communities[3]

People living in existing communities around the King's Cross Central site suffer from inequalities in the determinants of health.

Deprivation

- The King's Cross Central redevelopment sits on the boundary of two of the most deprived boroughs in London
- The level of deprivation experienced in the wards affected by the redevelopment of the King's Cross area is also high when compared with that in other boroughs throughout England

Employment, income and skills levels

- Income for about half the households in the King's Cross area is low – less than £200 a week
- There is a high level of dependency on means-tested benefit for residents in the wards affected by the redevelopment of King's Cross Central
- Unemployment in the wards affected by the redevelopment of King's Cross Central is high ranging from 7% to 13%
- Unemployment is even higher for black and minority ethnic groups in the community, particularly the Bangladeshi community
- There is a need for skills development in the communities surrounding the King's Cross Central site

Educational attainment

- For young people living in the King's Cross area, educational attainment is lower than that achieved nationally
- For a substantial number of adults in the area, educational attainment is also low – 40% of households in the King's Cross Household Survey, 2000, had poor or no GCSEs; this is particularly marked in the Bangladeshi community

English language and communication abilities

- More than 10% of local households have difficulties with speaking, reading, writing or understanding English; again, this is particularly acute for the Bangladeshi community, but during the HIA it has also been found in the Somali community and the Turkish-speaking community, especially for the elders

[3] Data taken from profile of community in the PCTs' response to Argent St George's Third Consultation Document, and Appendices 2 and 3 of the PCT's evidence presented on behalf of LBC at the Planning Inquiry into 24-hour working on the Channel Tunnel Rail Link (CTRL) Project

Housing conditions

- There is an acute need for housing in the King's Cross area - Camden is ranked as the 7th most crowded borough in England
- There is poor provision of suitable housing and accommodation in wards affected by the redevelopment of the King's Cross area when compared with other wards in Inner London, and those in England
- More than a quarter of the households in the King's Cross area are overcrowded, a condition that affects almost half of the Bangladeshi community in the area

Crime and community safety

- Violence against the person and drugs crimes give most cause for concern to the local communities in the King's Cross area
- A high proportion of violence against the person and drugs crimes in Camden are committed by visitors to the area or by people drawn to the area
- Racial harassment and violence are common in some parts of the King's Cross area, and engender feelings of fear and intimidation in people from black and minority ethnic groups living in the area

Access to and use of leisure facilities

- There are gaps in the provision of sports facilities in the King's Cross area
- The usage of leisure facilities tends to be lower in black and ethnic minority groups, and manual occupational groups in both Camden & Islington

Access to and use of green and open spaces

- There is a severe lack of green and open space in the King's Cross area
- There is a low usage of the existing green/open spaces, which may reflect a lack of accessibility

Access to transport

- Residents in the wards directly affected by the redevelopment of King's Cross Central have poor access to private transport (car)
- Access to public transport at King's Cross is generally good, but access to some forms of transport is better than others
- In recent years, access to public transport has been constrained by disruptions arising from the construction works associated with Channel Tunnel Rail Link (CTRL) and London Underground Ltd (LUL)

Access to affordable nutritious food

- There is a need for a shopping facility that provides access to affordable, good-quality, nutritious food in the King's Cross area; the Tesco Metro on the Caledonian Road will not meet this need

Access to healthcare

- Access to primary healthcare is poor in the King's Cross area, particularly for the black and minority ethnic communities

Box 2.3

Key facts about the health of existing local communities

People living in existing communities around the King's Cross Central site suffer from inequalities in health and well-being.

Mortality

- The main causes of death for Camden residents (2001-2002) were cancer (25%), coronary heart disease (18%), and respiratory systems disorders (12%).
- Mortality rate from all causes is higher in the St Pancras and Somers Town ward when compared with that for England (London Health Observatory, 2003).
- Mortality rate for cancer (1998-2002) for St Pancras and Somers Town ward was highest of all the Camden wards, and higher than that reported for London and for England (London Health Observatory, 2003).
- Mortality rate for circulatory disease (1998-2002) for St Pancras and Somers Town ward was second highest of all Camden wards, and higher than that for London and for England (London Health Observatory, 2003).
- Mortality rate for coronary heart disease (1998-2002) for St Pancras and Somers Town ward was fourth highest of all Camden wards, and higher than that for London and for England (London Health Observatory, 2003).
- Mortality rate for all causes (1998-2002) for St Pancras and Somers Town ward was second highest of all Camden wards, and higher than that for London and for England (London Health Observatory, 2003).
- Mortality rate for accidental injury was higher in the old Somers Town ward when compared with that for England & Wales.

Morbidity

Hospital admission rates per 100,000 persons can be taken as indicators of morbidity, or poor health, in the community.

Physical health

- Expenditure on prescribed drugs for respiratory disease is greater per patient for general practitioner practices in the wards affected by the redevelopment than those within Camden and Islington as a whole.
- Tuberculosis (TB) notification is higher in both Camden and Islington when compared with London and England.
- Admission rates (standardised) for injury were higher than those in England and Wales for the old Somers Town ward.
- The old Somers Town ward had the third highest age-standardised hospital admission rate for injuries of all Camden wards.
- In 2000, the percentage of low-weight births was higher in the St Pancras and Somers Town ward than in Camden overall, London or England.
- Admission rates per 1000 persons (crude) to Accident & Emergency for children aged 0 to 3 years were higher in the old Somers Town ward than those for Camden as a whole.
- Admission rates per 1000 persons (crude) to hospital for children aged 0 to 3 years were higher in the old Somers Town ward than those for Camden as a whole.
- Admission rates to hospital for lower respiratory tract infections per 1000 children aged 0-4 years in the St Pancras and Somers Town ward was highest of all Camden wards.
- The teenage pregnancy rate is higher in the St Pancras and Somers Town ward than that in Camden as a whole.

- In people of South Asian descent, there are higher rates of coronary heart disease and communicable disease notification than in the general population
- Tuberculosis notification rate for all minority ethnic populations in Camden and Islington is considerably higher than that for the White population (130 vs 17 per 100,000 people).

Mental health and well-being

- In the local boost to the Health Survey for England 1998, 23% of residents in Camden and Islington were found to have difficulties in coping with daily life to the extent that they may be suffering from psychiatric morbidity (GHQ[4] score of 4+) compared with 15% of people in England.
- Over 30% of residents in Camden and Islington on a low income (<£10,585 per year) have difficulties in coping with daily life compared with 14% on a high income (>£21,990 per year).
- In the local boost to the Health Survey for England 1998, 18% of residents in Camden and Islington were found to be experiencing a severe lack of social support compared with 14% of people in England.
- Over 30% of residents in Camden and Islington on a low income (<£10,585 per year) have a severe lack of social support compared with 8% on a high income (>£21,990 per year).
- Admission rates (standardised) for mental health problems are higher in the old Somers Town ward than those for Camden as a whole. In fact, the Somers Town ward has particularly high mental health admission rates.
- Expenditure on prescribed drugs for mental health is greater per patient for general practices in the wards affected by the redevelopement than those within Camden and Islington as a whole.

Self-reported health issues

- In the 2001 Census, the St Pancras and Somers Town ward reported the highest levels in Camden of all people with limiting long-term illness (LLTI), people of working age with LLTI, and people who reported their health as being "not good" (20%, 12%, and 9% respectively).
- In the King's Cross Household Survey 2000, 17% of households reported their health as "not good", and 6% reported it as "poor"; long-term residents, who tend to be older people, were more likely to report poor health.
- In the King's Cross Household Survey 2000, 29% of households had one member who was suffering from a long-term illness - the illnesses mentioned most frequently are:
  - breathing problems 30%
  - heart problems 28%
  - arthritis 18%
  - limb movement 16%
  - depression 14%
- Health status was reported as "bad" or "very bad" by 17% of Black and 15% of Asian residents in Camden.

[4] General Household Questionaire (GHQ12), which contains questions relating to coping with daily life.

Table 2.2: The likelihood of which positive and negative effects of redevelopment might occur at King's Cross

Positive effects	Likelihood of occurring at King's Cross?	Negative effects	Likelihood of occurring at King's Cross?
Physical renewal	Definite	***Displacement of residents***	Displacement of residents from privately rented accommodation
Increased tax revenues	Definite	***Displacement of businesses***	Definite, especially for small and medium-sized enterprises, and as land values, taxes, and rents increase
Reduced crime	Debatable – overall some crime could be displaced; there may also be changes in the nature and location of crime at King's Cross Central	***Increased crime***	Definite in areas to which traditional patterns of crime seen at King's Cross are displaced which are likely to be surrounding estates; increase in other crimes due to introduction of transport hub/port in vicinity
Improved local services	Depends on the provision of facilities and services and whether what is provided matches need and demand	***Changes to local services***	Some public services are currently over-stretched, e.g. health care – increased demand without increased provision will compromise both the volume and quality of services available to the whole community
Increased social mix	Increase in the number of relatively affluent and healthy people moving in to King's Cross Central	***Social displacement***	Displacement of extremely vulnerable, such as homeless and refugees & asylum seekers as amount of temporary accommodation in area is reduced
Increased population stability	Limited effect at King's Cross given the amount of housing available for social renting; however, there may be also be a destabilising influence from an influx of transient population from Europe due to the termination of Eurostar at St Pancras; there may also be a greater influx of homeless people from Kent	***Community conflict***	Definite if people in social housing on and around site do not benefit from jobs and other opportunities created but instead are subject to increased cost of living and reduced disposable income, which will have negative effects on health and well-being, and accentuate any differences between them and the more affluent who are able to advantage of the opportunities

2.3 Modelling the potential impacts of the major deliverables from the redevelopment of King's Cross Central

In order to assess what might be the likely impacts on existing local communities living around the King's Cross Central site, we have constructed a model of the potential pathways of positive and negative effects based on the major deliverables from the redevelopment:

- an increase in the number of employment opportunities;
- an increase in the number of residential units.

This model is shown in Figure 1. From this figure, it can be seen that there are three major changes that redevelopment will bring to King's Cross:

- an influx of workers from outside the area;
- an influx of regional, national and international businesses;
- an influx of new residents, most of whom will be affluent and healthy.

An ***influx of workers*** from outside the area could mean:

- there will be a decrease in the number of job opportunities available for local people;
- low-quality jobs will be available to local people;
- local people will be on low pay.

An ***influx of regional, national and international businesses and chains*** could mean:

- an increased cost of living at King's Cross;
- a reduced level of disposable income for local people;
- a reduction in the retention of money in the local people;
- displacement of voluntary sector organisations from the area;
- a loss or reduction of voluntary sector services.

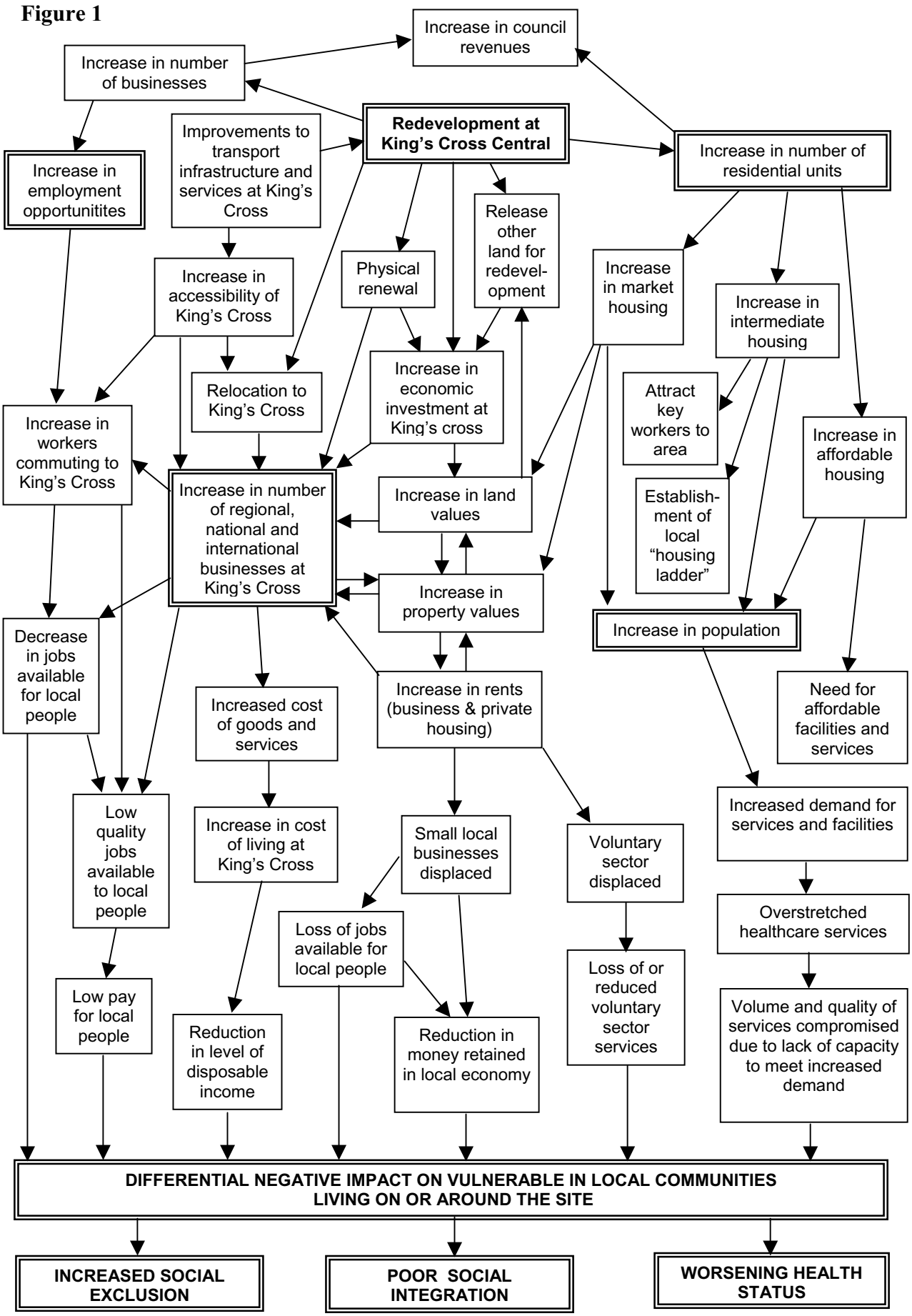
An ***influx of new residents***, most of whom will be affluent and healthy, could mean:

- increased demand for services and facilities;
- need of local people in existing communities for affordable services and facilities;
- lack of capacity in some services and facilities to meet increased demand, e.g. health care, leading to compromised service provision in terms of volume and/or quality.

For the people already living around King's Cross, most of whom are living in housing for social renting and are vulnerable in terms of health and other inequalities, it can be seen that the redevelopment at King's Cross Central, particularly with respect to jobs and housing, could have a differential negative impact, leading to:

- increased social exclusion;
- poor social integration;
- worsening health status.

Figure 1

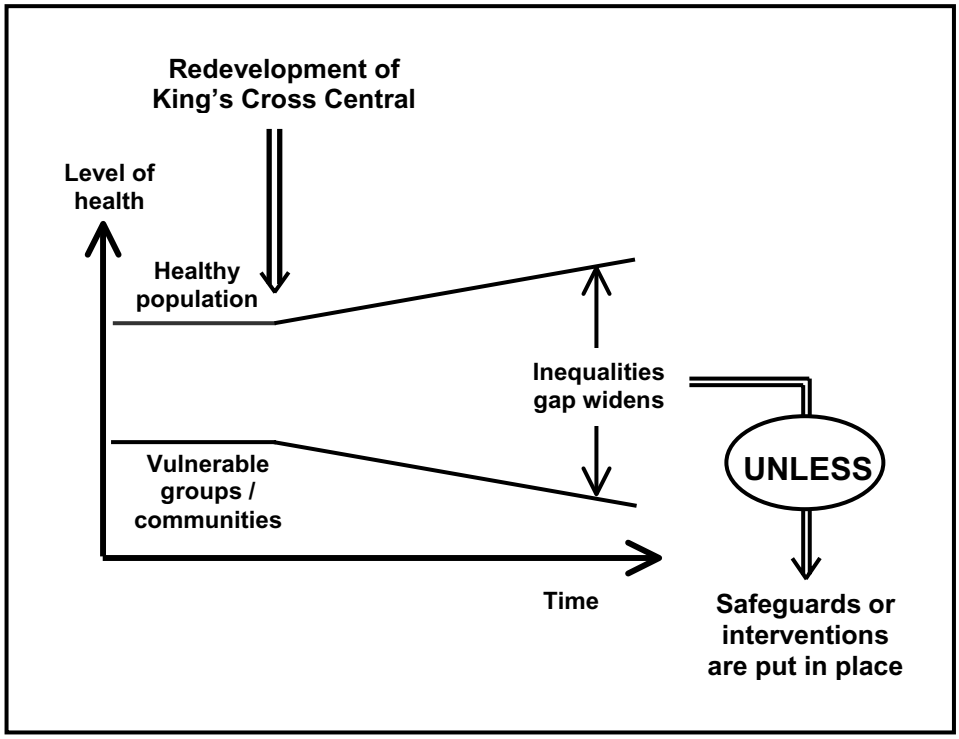

Increase in council revenues
Increase in number of businesses
Redevelopment at King's Cross Central
Improvements to transport infrastructure and services at King's Cross
Increase in number of residential units
Increase in employment opportunitites
Release other land for redevel-opment
Physical renewal
Increase in accessibility of King's Cross
Increase in market housing
Increase in intermediate housing
Increase in economic investment at King's cross
Relocation to King's Cross
Attract key workers to area
Increase in workers commuting to King's Cross
Increase in affordable housing
Increase in land values
Establish-ment of local "housing ladder"
Increase in number of regional, national and international businesses at King's Cross
Increase in property values
Increase in population
Decrease in jobs available for local people
Increase in rents (business & private housing)
Need for affordable facilities and services
Increased cost of goods and services
Increased demand for services and facilities
Low quality jobs available to local people
Increase in cost of living at King's Cross
Small local businesses displaced
Voluntary sector displaced
Overstretched healthcare services
Loss of jobs available for local people
Loss of or reduced voluntary sector services
Low pay for local people
Reduction in level of disposable income
Reduction in money retained in local economy
Volume and quality of services compromised due to lack of capacity to meet increased demand
DIFFERENTIAL NEGATIVE IMPACT ON VULNERABLE IN LOCAL COMMUNITIES LIVING ON OR AROUND THE SITE
INCREASED SOCIAL EXCLUSION
POOR SOCIAL INTEGRATION
WORSENING HEALTH STATUS

In Figure 2, we show that this series of differential negative impacts could serve to increase the inequalities gap UNLESS safeguards and interventions are put in place to prevent this from happening. It is not enough to rely on the protective factors mentioned by Argent St George (physical form, social structure, and landownership patterns).

The types of safeguards that need to be put in place are discussed in the next section, Section 3, under headings relating to the determinants of health and well-being.

It is important to note here that Argent St George do not specifically address the inequalities gap. In fact, in the Environmental Statement, they have marginalized the impacts on vulnerable groups in the population by assessing impacts of "minor" significance if they affect only a relatively small number of people, i.e. those living in the Central Impact Zone as defined by Argent St George. This is to nullify the impacts on the most vulnerable people living in the communities around the King's Cross Central site, which could lead to the impacts on them being ignored, and/or not addressed effectively or appropriately. For a more detailed discussion of the assessment of significance, see Part III of the Response – Commentary on Chapters 12 and 13 of Argent St George's Environmental Statement.

Figure 2



Section 3: Potential impacts of the redevelopment of King's Cross Central on health and well-being

In Section 3, we assess the impacts of the redevelopment of King's Cross Central on various groups of people who will be affected by the development.

The key groups of people we will be considering are:

1. existing local communities around the site – this is similar to residents living in Argent St George's Central Impact Zone;
2. new residents on the King's Cross Central site;
3. workers on the King's Cross Central site;
4. passengers using the major transport hub at King's Cross, when it is appropriate to consider them.

We may also consider visitors to the site when relevant.

In Table 3.1, we present information on the key deliverables from the redevelopment of the King's Cross Central site:

- Economic investment;
- Employment opportunities;
- Housing;
- Potential new facilities;
- Potential new amenities;
- Green and open spaces, and new habitats.

We discuss the potential impacts on health and well-being under a series of headings reflecting key determinants of health. For each determinant of health, we have adopted the same series of subheadings to give this section of Part I consistency, as follows:

- What will the King's Cross redevelopment deliver?
- Who will benefit and how?
- What will be the effect on the inequalities gap?
- How can we ensure that benefits can be realised?
- Key questions that need to be addressed

An important influence on the way a proposal, or certain elements of it, can affect the health of a community, or groups of people within it, is the level of pre-existing health and other inequalities. For instance, a proposal that may have a positive or neutral effect on healthier people in the community could have a negative effect on vulnerable people within that same community. Alternatively, the degree of impact may be different, vulnerable people may experience a greater potential negative impact than healthier ones who are better able to withstand or deal with any potential negative effects.

As the communities living around the King's Cross Central site experience many health and other inequalities (see Boxes 2.2 and 2.3), in this section, we have assessed, under headings for each determinant of health, whether elements of the redevelopment at King's Cross will have differential effects for the vulnerable in the

surrounding communities, and whether certain deliverables narrow, widen, or may not affect the inequalities gap.

Table 3.1: What are the main deliverables or outputs from the redevelopment of King's Cross Central (KXC)? *NB: Information sourced from ASG documents*

Economic development Investment in the following categories of land use: ▪ business & employment – up to 250 businesses by 2020; ▪ residential; ▪ shopping, and food & drink; ▪ hotels /serviced apartments ▪ assembly and leisure; ▪ health, education, community and cultural; ▪ public realm – streets, parks, squares, including green space and habitats.	***New housing*** An increase in the number of residential units ▪ 1600-2300 on Main site ▪ up to 250 on Triangle site *Affordable housing* ▪ 30-50% on Main site ▪ 30% = 480-800 units ▪ 50% = 690-1150 units ▪ up to 35% on Triangle site – up to 87 units *Types of affordable housing* ▪ For social renting ▪ For intermediate occupiers – "high" proportion NB: Percentage split not known for types of affordable housing
Employment An increase in the number of employment opportunities *Operation of KXC*[5] Minimum = 16,515 ▪ CIZ residents = 825 ▪ WIZ residents = 4128 Maximum = 19,664 ▪ CIZ residents = 983 ▪ WIZ residents = 4916 *Construction of KXC* ▪ Construction = 2670 ▪ CIZ residents = 134 ▪ WIZ residents = 668 *Multiplier effect: operation of KXC* Minimum = 24,773 ▪ CIZ residents = 1239 ▪ WIZ residents = 6193 Maximum = 29,496 ▪ CIZ residents = 1475 ▪ WIZ residents = 7374 *Multiplier effect: construction of KXC* ▪ Construction = 3005 ▪ CIZ residents = 150 ▪ WIZ residents = 752	***Potential new facilities*** *Health care* ▪ A medi-centre in Zone W, Triangle site ▪ Possibility of a walk-in centre – public or private *Education* ▪ Higher education facility in Zone L *Leisure* ▪ A leisure centre on the Triangle site with a sports hall or swimming pool ▪ A range of health clubs & other indoor sports facilities ▪ Ice-skating opportunities ▪ Scope for rock-climbing, 'bouldering' or more challenging activities ▪ Boule, tennis, wall tennis and areas for 5-aside football ▪ A cinema ▪ Play areas for 1-8 year olds ▪ Places for 8-16 year olds to play
Potential new amenities ▪ Shops ▪ Supermarket in Zones L, P/S, and/or W ▪ 50 Food & drink outlets ▪ A multi-storey car park in Zone T	***Green spaces/habitats*** *Green spaces* ▪ Introduction of the Long Park *Habitats* ▪ Introduction of habitat in Zone W, Triangle site

[5] Figures take into account displacement

3.1 Employment opportunities

Policy/Strategy drivers

"Successful businesses need well-educated, well-trained staff. We want them to employ local people – if our residents are able to work, we will help them get work. We will make sure our residents benefit from the economic success of the borough."

London Borough of Camden, Community Strategy, June 2001

"… with the right mix of local opportunities and support people facing multiple disadvantage and deprivation will at least be able to find work and lift themselves out of poverty."

London Borough of Camden, Social Inclusion Strategy, August 2003

3.1.1 What will the King's Cross Central redevelopment deliver?

The employment opportunities Argent St George estimate that the redevelopment of the King's Cross Central site will deliver are shown in Table 1. Argent St George have calculated the number of jobs that will be generated both with and without the multiplier effect. They have also assumed that 5% of these jobs will go to residents in the Central Impact Zone (CIZ)[6], and 25% will go to residents in the Wider Impact Zone (WIZ)[7].

The King's Cross HIA Team have undertaken an analysis of the phasing and completion of the various development zones in order to determine how many job opportunities will be available when. This is a difficult task because any estimates have to be based on the figures presented by Argent St George, which are limited to the maximum floor space allocated to business & employment, and other uses, in each development zone, and do not necessarily represent what will be built, and therefore do not reflect the true number of job opportunities available (they are overestimates).

In addition, in our analysis, we have not taken account of the multiplier effect because it is not clear when jobs arising from any multiplier effect will become available in relation to the phased completion of the development zones. Some of these jobs could be generated not long after the jobs on site, but others may not be available until some time after jobs have been generated on site (it is not possible to work out this time-lag).

However, the analysis the King's Cross HIA Team has undertaken of the phasing of completed development zones shows that:

- the earliest time-point at which the first development zones will be completed that are anticipated to generate several hundred jobs – about 281-301 – for local residents (i.e. those living in Central Impact Zone) is 2013-2014; this will be seven years after the earliest start date for construction of the redevelopment (see Table 12, in Part II of the Response);

[6] Central Impact Zone includes people living in the following wards: Caledonian, King's Cross, and St Pancras and Somers Town,

[7] Wider Impact Zone includes people living in the following wards: Barnsbury, Bloomsbury, Clerkenwell, Cantelowes, Holborn & Covent Garden, and Regent's Park,

- the next time-point at which a similar number of jobs for local residents (in CIZ) – about 345 – will be generated by a group of completed development zones is towards the end of construction in 2017-2019 (see Table 12, in Part II of the Response).

Thus, when put into the perspective of the number of jobs delivered for local people (in CIZ) over the timeframe of the redevelopment's phased construction and eventual completion 12 (or more) years later, the number of jobs for local people is relatively small and spread out over a long time period.

Argent St George have also estimated that local people (in CIZ) will obtain 134 construction jobs, although these employment opportunities will be spread out over the timeframe of the redevelopment's phased construction. It is not clear how many of these jobs will be available at what time-points.

Box 3.1 ***Employment opportunities***

What does the evidence base show?

- Job creation does not necessarily "trickle down" as job opportunities for the long-term unemployed, and it is not a sufficient or necessary condition for reducing long-term unemployment (Campbell et al, 1998).
- Employment does not necessarily lead to health improvement - the health consequences of employment are directly related to the quality of work (Graetz, 1993).
- Employment opportunities created by regeneration schemes risk being dominated by low-paid, insecure, secondary sector, non-standard forms of employment – i.e. jobs of low quality (Deakin, 1995; Peck and Theodore, 2000; Martin et al, 2000).
- A transition from unemployment to "inadequate" work is unlikely to be beneficial to health (Dooley et al, 2000; Halvorsen, 1998; Ezzy, 1993; Fryer, 2000).
- Re-employment in low-quality work may be worse for psychological health than the experience of unemployment (Dooley et al, 2000; Burchell, 1994; Winefield et al, 1993).
- New jobs created by regeneration initiatives are often filled by workers from other parts of the city rather than local populations in areas targeted for regeneration (Brownhill, 1999; Bailey and Turok, 2000; Turok and Webster, 1998).
- Education, training and learning play important roles in providing the basis for economic growth (Organisation for Economic Co-operation and Development – OECD, 2001).

What does the local information base show?

"… the local jobs market poses particular problems for joined-up interventions by the Council and its key partners. … While employment in Camden has risen with national trends over the last decade, the types of jobs emerging require higher skills and qualifications than before. Despite Camden having high levels of people with higher qualifications and degrees this skills challenge has not been met across the board, resulting in a demonstrable shift towards skilled workers commuting in from the suburbs and outside London to satisfy this demand. Camden, with its rich diversity of knowledge economy firms and major mainline stations located in our most deprived wards, experiences this phenomenon on a daily basis."

London Borough of Camden, Social Inclusion Strategy, August 2003

3.1.2 Who will benefit and how?

It is highly likely that the major beneficiaries from the employment opportunities on offer at King's Cross Central will be:

- workers from other parts of the borough, city or wider region – this effect will be magnified by the enhanced transport accessibility at King's Cross Central arising from the completion of the major transport infrastructure projects in 2007.

New residents in the market housing may also be beneficiaries of the employment opportunities on offer.

It is likely that both these groups of people will already enjoy a relatively good state of health and well-being. The employment opportunities available to them at King's Cross Central will probably serve to maintain if not improve it.

Some people in the existing local communities around the site – approximating to Argent St George's Central Impact Zone - may obtain work at King's Cross Central. Argent St George's estimate is that they will be able to take up 5% of the employment opportunities on offer.

If this assumption is correct, this might help to reduce the high unemployment rate in the local community, which would have a slightly positive impact in the local economy and benefit the families concerned. However, the jobs taken up by local people may not go to those who are unemployed, and in poor health, but to those who already have jobs, and are likely to be in better health.

In addition, as Argent St George states that the levels of uptake of job opportunities by local people (5 and 25% by people in CIZ and WIZ) will be possible ***without*** the need for employment and brokerage schemes, it is highly likely that the quality of jobs on offer to local people (in CIZ) will be low, especially given the low level of skills and educational attainment for adults and some young people in this area. As can be seen from the evidence base (see Box 3.1), low-quality work is unlikely to be beneficial to health, and may even be worse for mental health than unemployment.

3.1.3 What will be the effect on the inequalities gap?

Without skills development, training and education, and employment brokerage schemes for local people (particularly long-term unemployed and those with low skills/poor qualifications), it is likely that the inequalities gap will widen: many of the new residents on site in the market housing will be healthy people and some of the jobs on offer will go to local people who are already in employment and are likely to be healthier. Although at least some local people will obtain employment, those who are unemployed or vulnerable in other ways will probably be able to access only the low-quality jobs on offer, which will not close the inequalities gap and could make it worse (see evidence base in Box 3.1).

3.1.4 How can we ensure benefits are realised?

Policy/Strategy Drivers

'Jobs for local people' is one of four main priorities for the Local Health Partnership (LHP) in Camden.

"The road to paid work for the long-term unemployed is challenging and requires highly committed multi-agency approaches, linking employers to trainers and public services. It also requires a long-term commitment as people who have been out of work for some time require a significant amount of support to re-enter the job market. Providing more training and employment for local residents in high-risk groups is key.

Camden Council is committed to using all the levers at our disposal to try and provide employment opportunities for local people who are long-term unemployed. …As well as using our own recruitment policies, we will try to achieve tangible results by using opportunities like the redevelopment at King's Cross, …"

London Borough of Camden, Social Inclusion Strategy, August 2003

Camden and Islington expect at least 15% of new jobs to be taken up by local people by 2012, linked to phasing, education and an ongoing employment strategy.

2.8.3, Joint Planning and Development Brief

We endorse Argent St George's support for:

- construction training projects;
- an employer-led Local Employment Strategy;.

However, we feel that Argent St George need to give more than simply their support for the principles of job brokerage initiatives, especially as these initiatives could be instrumental in obtaining jobs for local people.

We support Argent St George in their intention:

- to give early support to employment brokerage and skills training initiatives that match identified employers' needs with skills shortages or residents' aspirations;
- to encourage occupiers to engage with local employment initiatives in the manner most appropriate to them.

However, we ask that:

- Argent St George commit themselves to a King's Cross Education, Employment and Training Strategy as outlined in Section 2.8.3 of the Joint Planning and Development Brief
- Argent St George explore the issue of employing local people when negotiating with potential occupiers of King's Cross Central;
- any employment brokerage schemes for local people coincide with the phasing of development zones on the Main site, such that local people can try for the employment opportunities as they are arise;
- recruitment by all business occupiers at King's Cross Central is monitored to ascertain whether local people are gaining jobs, and if not, why not;

- Argent St George address the content of section 2.8.3 in the Joint Planning and Development Brief and strengthen the Code of Construction Practice (COCP) to include measures for local recruitment - as it stands, COCP only mentions advertising jobs locally
- any training and skills development programmes for local people need to address barriers to course uptake, e.g. by providing access to crèche or day care facilities, by using local venues as training locations, ensuring the affordability of any training schemes, and publicising the courses effectively, i.e. information available at points in the community not just at Job Centres;
- effective training courses in English language skills need to be developed and targeted at people in the black and minority ethnic communities, e.g. Bangladeshi, Somali, and Turkish speakers, to address the deficit in English language skills, which may increase their chances of obtaining employment, of obtaining better-quality employment or of being able to attend a further training course to improve their employment prospects.

3.1.5 Key questions that need to be addressed

- Argent St George have referred to the number of jobs generated at King's Cross Central as job opportunities, and termed this aspect of the redevelopment "job creation". However, how many of these job opportunities could be "lost" if regional, national and international companies (i.e. some of the potential occupiers Argent St George wish to attract, especially to the southern part of the site next to the transport terminals where the largest concentration of office space is to be built) re-locate to King's Cross Central and thereby take a certain proportion of staff with them. It is also more likely that the senior or skilled staff in an organisation will re-locate rather than staff in lower-paid, less-skilled work. Thus, the job opportunities on offer from any companies that re-locate are likely to be less than the number originally anticipated by Argent St George (based on floor space), and also more likely to be of lower quality.

- What is the basis for assuming that residents in the Central Impact Zone will be able to obtain 5%, and residents in the Wider Impact Zone 25%, of the employment opportunities provided by the redevelopment? In the Environmental Statement (page 12-42, paragraph 12.7.42), Argent St George do not give evidence to support these assumptions.

- It is not clear how Argent St George have calculated the multiplier effect of additional jobs generated as a result of spend from new and increased wages of people working at King's Cross Central and from linkages with existing and new services and supply businesses. This means it has not been possible to assess whether the figures Argent St George quote for the number of job opportunities available overall and for local people are realistic and capable of realisation.

- What interventions would be necessary to gain 40% of job opportunities for local people (cited in Environmental Statement and Regeneration Strategy)?

- How many jobs will go to the long-term unemployed (with and without interventions)?

3.2 Housing

3.2.1 What will the King's Cross Central redevelopment deliver?

Policy/Strategy drivers

"There's not enough good quality affordable housing in the borough. This leads to a number of problems. For example, people working in Camden such as teachers and nurses often can't afford to buy or rent a home in the borough, and they don't qualify for a council or a housing association home. It means that people on the Council's waiting list and people who are homeless or living in overcrowded conditions have to wait longer for a home, or have less choice. It can also mean that the voluntary sector and businesses get priced out of the areas that they need to work in."

London Borough of Camden Community Strategy, June 2001

"Initiatives to increase housing supply in Camden over the next few years include the requirement of 50% of new homes in the King's Cross Development to be affordable"

London Borough of Camden, Social Inclusion Strategy, August 2003

The numbers of residential units Argent St George estimate the redevelopment of the King's Cross Central site will deliver are shown in Table 3.1.

The King's Cross HIA Team have undertaken an analysis of the phasing and completion of the various development zones in order to determine how many residential units will be available when. This is a difficult task because any estimates have to be based on the figures presented by Argent St George, which are limited to the maximum floor space allocated to residential uses in each development zone, and do not necessarily represent what will be built, and therefore do not reflect the true number of residential units available (they are overestimates). In addition, as Argent St George have not indicated how many of what size of units (in terms of number of beds) will be built in each zone, we have had to estimate the number of units available at each time-point using an average floor-space per unit.

From the analysis undertaken by the KX HIA Team:

- the first group of development zones to be finished that will contain residential units is scheduled for completion in 2012-2013, with 161 residential units, 40-80 of which could be affordable;
- in 2013-2014, development zones containing relatively large numbers of residential units - 682 in total - are due for completion and the affordable housing ranges from 204 to 342 units;
- the triangle site is scheduled for completion in 2014-2015, bringing online a maximum of 250 units of which 87 may be affordable;
- the next year, 2015-2016, will see a further group of zones completed containing 347 units, of which 140-171 could be affordable;
- finally, over the time period 2017-2019, development zones containing the largest number of residential units will be completed, yielding 1151, of which 345-576 could be affordable.

3.2.1.1 Trade-offs

From Table 3.1, it can be seen that Argent St George have incorporated flexibility into the number of residential units they might build by stating a maximum and a minimum number for the Main site. The number of units for the Triangle site is given as a maximum of 250 without indicating what the minimum might be.

See Section 6 of Part II of the Response for an analysis of the trade-offs involved, but it seems that the major trade-off with respect to determining how much housing is built is office space, and it is highly likely that Argent St George will build more offices and less houses.

However, Argent St George have incorporated two further degrees of flexibility within the residential land use category:

- a trade-off ***within*** the category of residential floor space between market housing and affordable housing;
- a trade-off within the category of affordable housing between housing for social renting and intermediate housing.

With respect to affordable housing, Argent St George have calculated the amount that could be delivered on the Main site as a minimum of 30% and a maximum of 50%. If the minimum of 1600 residential units was built, the difference between the maximum and minimum number of affordable units would be 320 (26 units a year over 12 years), and if the maximum of 2300 residential units was built, the difference between the maximum and minimum number of affordable units would be 460 (38 units a year over 12 years).

It is likely that Argent St George will want to negotiate for a lower level of affordable housing in relation to market housing than the maximum of 50%:

- to maximise their revenue potential from the site;
- in order, in their words, "to rebalance the tenure profile" in the area and provide the basis for a local housing ladder – Argent St George's contention is that within 1 mile of the site the proportion of affordable housing is 70%. We are not sure of the evidence to support this figure, however, data from the LASIR Project (funded by Camden PCT, Islington PCT, and the Metropolitan Police) suggests that the proportion of affordable housing in Camden within a 500-metre buffer zone around the King's Cross Central site is 58%, with private dwellings accounting for the other 42%.

With respect to the proportion of housing for social renting and that for intermediate occupiers, Argent St George have not stated what proportions they intend to build, although they appear to favour a relatively high proportion of intermediate housing as another way of "rebalancing the tenure profile" and introducing a local housing ladder (cited in the Regeneration Strategy and in the Environmental Statement).

Box 3.2 ***Housing conditions***

What does the evidence base show?

- Poor housing may affect physical health (Acheson et al, 1998; Lowry, 1991; Best, 1995; Best, 1999; Burridge and Ormondy, 1993; Bardsley et al, 1998; Victor, 1997) in the following ways:
  - it increases the risk of injury;
  - if conditions include cold, damp or mould, it increases the level of reported respiratory disease and gastro-intestinal problems;
  - if conditions include overcrowding, it increases the risk of infections especially in temporary accommodation;
  - for children living in temporary accommodation, it increases susceptibility to disturbed sleep, poor eating, hyperactivity, bedwetting and soiling, aggression, and higher rates of accidents and infectious disease.
- Mental health can be negatively affected by living in cold, or damp conditions or crowded housing in poor condition (Acheson et al, 1998; Lowry, 1991; Best, 1995; Best, 1999; Burridge and Ormondy, 1993; Bardsley et al, 1998; Victor, 1997).
- Living in cramped unsanitary conditions is very stressful and undermines personal and parental relationships (British Medical Association, 1987).

Housing design

- In British women, there is a suggestion that high-density living may have a damaging effect on mental health (Gabe and Williams, 1993), but the design of housing may not be solely responsible for this link (Hunt, 1997).
- Residents of high-rise flats report a greater sense of isolation and have a higher risk of suffering from psychiatric disturbance than people who live in houses (Seymour et al, 2001).

Housing regeneration

- Housing regeneration programmes do no always improve housing equally for all local populations. Those with the most severe housing needs and some marginalised groups may be ***further excluded*** or ***displaced*** by housing schemes (Curtis et al, 2002).

Mixed tenure

- Mutual residence between owner-occupiers and those in affordable housing does not of itself bring people together (Atkinson and Kintrea, 2000). Home-owners can make a contribution to transforming deprived communities but change may be slow and the outcomes uncertain depending on the context (spatial and social) in which mixed tenure policies are implemented.

What does the local information base show?

- The shortage of affordable housing was cited as the most apparent health care need in Camden Central, and probably the main cause of stress for council tenants (Keenan, 2002).
- From the Community Workers in HIA Project, two communities in particular need of affordable housing are the Bangladeshi and Somali.
- An increasing number of families are being accommodated in an area with a shortage of family housing or outside space, resulting in children being brought up in cramped, noisy surroundings which exacerbates the spread of infection (Di Janus, 2000).

3.2.1.2 Mixed tenure

Argent St George, to our knowledge, have not indicated whether the residential units provided in each of the relevant blocks or buildings in development zones on the Main site will be of mixed tenure. However, in the Triangle Explanatory Statement (page 5), they do "anticipate" that of the two blocks allocated to residential use Block A (parallel to the Thameslink tracks) will be "given over" to market housing, and Block B (fronting York Way) would accommodate affordable housing. This strategy would split the affordable from the market housing. It is possible that Argent St George plan to replicate this strategy on the Main site.

3.2.2 Who will benefit and how?

Argent St George's intention to build a minimum of 1600 residential units is to be welcomed (with reservations) in a borough that has an acute housing shortage. However, if this minimum amount of residential units is to be built, it would only fulfil 9.4% of new homes required in Camden before 2016 according to targets set in the London Plan 2004.

The greater the proportion of market housing in the development, i.e. up to 70%, it is certain that the greatest number of people obtaining most benefit from the residential units will be new residents moving into the area who are likely already to be experiencing good health and well-being. As the amount of market housing on site increases, the number of people in the local community and wider borough who are in need of housing or re-housing, and likely to be in poorer health who could experience benefits from better housing conditions, will decrease.

For those among the vulnerable who are housed/re-housed, better housing conditions will halt a decline in health, and help people to maintain their current health status. However, to obtain improvements in health, it is necessary to ensure improvements in other health determinants such as employment, level of income, crime and community safety, and environment at the same time as delivering improvements in housing. There is the potential to provide this package of benefits at King's Cross Central, but it is not certain that some of those housed/re-housed will be able to take up the employment opportunities available.

As the amount of affordable housing built for intermediate occupiers increases, the amount of housing for social renting decreases. Consequently, the number of vulnerable people who could have the decline in their health halted and stabilised will be reduced.

For people who would qualify as intermediate occupiers, including key workers, an increase in the proportion of intermediate housing increases their chance of entering the housing market. Housing for key workers may be a factor in attracting and trying to retain staff in public sector services, such as healthcare workers, teachers and police, which could improve the quality of services delivered to the local community, whether existing or new residents.

Although it is not clear what Argent St George intend for the Main site, for the Triangle site, the separation between two blocks or buildings of market and affordable housing reduces the opportunities for social contact between different types of residents, thereby reducing the possibility of social integration and community

cohesion. This strategy underlines a division, rather than seeking to reduce it, and could introduce stigma for residents in the affordable housing.

3.2.3 What will be the effect on the inequalities gap?

As the amount of market housing in the redevelopment increases, and the amount of affordable housing decreases, the inequalities gap will widen, with the health of the vulnerable, such as those who remain homeless (e.g. those in temporary accommodation) or who are living in overcrowded conditions, continuing to decline in contrast with an influx of healthy new residents in the market and intermediate housing.

If affordable housing is to be separated from market housing in discrete blocks on the King's Cross Central site, then the residents in the affordable housing may be stigmatised, which will affect their mental well-being, and this will serve to increase the inequalities gap.

3.2.4 How can we ensure the benefits are realised?

We support:

- London Borough of Camden's Unitary Development Plan Policy KC4 of 50% affordable housing for the first additional 1000 residential units to be built on the Main site at King's Cross Central – this would generate 500 affordable homes;
- London Borough of Camden's targets of 35% social housing for renting and of 15% for essential workers and other intermediate occupiers for the first additional 1000 residential units to be built on the Main site at King's Cross Central – this would generate 350 homes for social renting and 150 for intermediate occupiers. For units over the first additional 1000 units on the main site, 50% affordable housing comprising a significant element of social housing for renting and housing for essential workers (may also include other forms of low cost housing);
- London Borough of Islington's policy in the Unitary Development Plan of up to 35% of affordable housing on the Triangle site, but would urge for the maximum of 35% to be negotiated – if so, this would generate up to 87 units of affordable housing.

3.2.5 Key issues to be addressed

Argent St George have set forth arguments in the Regeneration Strategy of the need:

- to reduce the amount of affordable housing on the King's Cross Central site;
- to reduce the amount of housing for social renting in relation to intermediate housing with respect to units built after the first 1000.

Argent St George contend that a high proportion of affordable housing, and in particular housing for social renting, places unsupportable demand on local services. While this may seem to be a logical consequence at first sight, on closer examination the situation is not as clear-cut. For instance, the people who are moved into any social housing for renting on the King's Cross Central site are already receiving services from the borough. In fact, if a family is in temporary accommodation it may actually reduce demand on services when they are re-housed. Thus, for council services it may not be the case that there is greater demand on services/resources. However, for health services, and possibly the voluntary sector, there may be a shift

in resources locally when families/individuals are re-housed. Ultimately, there is the possibility that being re-housed not only helps to improve someone's health but also acts to reduce the level of resources needed to support them in the future.

Clarification is needed from Argent St George about whether they intend, on the Main site, to place affordable housing in buildings or blocks separate from buildings or blocks containing the market housing. This would appear to be the case for the Triangle site. In the Joint Planning and Development Brief (2.9.17) it states that affordable housing should not be located in discrete areas but integrated into the wider development with market housing and other uses.

To date, we have not found any mention of the provision of sheltered and residential accommodation in the planning applications or supporting documents despite this need having been highlighted in the Joint Planning and Development Brief (2.10.7).

The relatively low proportion of family-sized accommodation (~20% on the main site) to be built in comparison with the need for this type of accommodation in Camden.

Confirmation is needed from Argent St George about the redevelopment of the Triangle site (see Section 1.6 in Part III of this Response). If the Triangle site is not to be redeveloped there will be a loss of residential units of up to 250, including a loss of up to 87 units of affordable housing. Such a loss would reduce the capacity of King's Cross Central to generate health improvement.

If the Triangle site is to be redeveloped, the amount of housing, and affordable housing, that will be built in this zone (W).

3.3 Access to healthcare facilities and services

3.3.1 What will the King's Cross Central redevelopment deliver?

Argent St George anticipate that the maximum number of new residents living at King's Cross Central will be 5125 if 2300 residential units are built on the Main site and 250 are built on the Triangle site. The minimum number anticipated is 3338 if only 1600 units are built on the Main site and 250 units are built on the Triangle site.

When the redevelopment is completed, the maximum number of workers at King's Cross Central is estimated to be 19,664, and the minimum 16,515.

Arup have estimated there will be about 5 million visitors to the site a year (see Retail Impact Assessment, page 72).

In addition, it is estimated that from June 2007 there will be about 60 million passenger-journeys a year at the transport interchange at King's Cross.

These increases in number of residents, workers, and passengers will increase the demand for services, including health services.

Argent St George have made provision for D1 land uses, which includes health, in every group of development zones scheduled for completion at the same time, i.e. 2012-2013, 2013-2014, 2014-2015, 2015-2016, and 2017-2019. However, the only zone in which they specifically mention the construction of a "medi-centre" is Zone W, the Triangle site, which is scheduled for completion in 2014-2015. In Argent St George's third consultation document, it was suggested that healthcare uses could be provided in Regeneration House, which is part of Zone K and in the planning application this has provision for D1 land uses.

From the analysis of the KX HIA Team (see Table A10 in Part II of the Response), it is necessary to ensure that appropriate primary care facilities are provided on site as the redevelopment progresses in order to provide services for the increasing number of new residents living in recently built residential units. For instance, in 2012-2013, with the completion of two groups of zones, about 160 residential units will be provided, and a further 680 units will be provided by 2013-2014. It is important therefore to ensure that new healthcare facilities are provided for these residents otherwise other primary healthcare facilities in the vicinity may become overloaded by increased demand.

Similarly, it is important that any facility, such as walk-in centre, to accommodate the healthcare needs of new workers, passengers, and visitors/tourists to the site is built as the redevelopment progresses and therefore can meet any demand without putting pressure on services such as Accident and Emergency at University College London Hospital.

Box 3.3 ***Access to healthcare facilities and services***

What does the evidence base show?

- Communities at most risk of ill health tend to experience least satisfactory access to the full range of preventive services, including screening programmes for various cancers, health promotion, and immunisation (Goddard and Smith, 1998). These inequities in access are most notable in access over the range of socio-economic groups

What does the local information base show?

- There are particular concerns about the pressure of increased demand, arising as a result of the redevelopment, on local health services, whether providing primary or secondary health care. It is vital to address this issue because it is clear from the review of local health needs assessments that current levels of health need are not being met effectively or appropriately in some cases, and would be further compromised by increased demand.
- 9 of the 19 general practices in the area fall below the minimum standard for GP surgery premises (defined by NHS Statement of Fees and Allowances), and 8 of these have been assessed as being structurally incapable of being improved.
- Community and public sector professionals have identified the need for dental services to be developed in the King's Cross area, especially as there are many children in the area in need of access to NHS dental services, and there will also be an increase in population from the new residential units.
- Public sector professionals have identified the need for:
  - optometry services to be developed in the King's Cross area;
  - pharmacies in King's Cross Central.

3.3.2 Who will benefit and how?

Any residents in the area, whether new or in existing local communities, would benefit from a healthcare facility providing primary care and related services. However, those who are vulnerable in the community, e.g. residents in housing for social renting, will benefit more from improved access to healthcare facilities and services than those new residents in market housing who will probably be experiencing good health anyway.

Any residents in the area could benefit both directly and indirectly from the provision of a walk-in centre provided by the public sector (indirectly in that demands would not be made inappropriately on other services, which could be compromised in volume and quality for local residents who need them). However, if the walk-in centre were to be run by a private healthcare organisation, it would benefit only those who could afford to pay for services, which would not include vulnerable groups in the area.

3.3.3 What will be the effect on the inequalities gap?

The provision of accessible and high-quality primary care facilities and related services would help to reduce the inequalities gap by improving the health of vulnerable groups. In fact, in the short term, one of the quickest ways to reduce health inequalities is to provide access to good-quality healthcare; in the long term, positive changes in the determinants of health will improve the health of vulnerable groups.

If the range of healthcare facilities on offer at King's Cross Central included a walk-in centre it would help to reduce the inequalities gap if it was provided by the public sector because it would be accessible to all. However, if the walk-in centre was provided by the private sector, it would not reduce the inequalities gap because accessibility is limited to those users who could afford to pay.

3.3.4 How can we ensure the benefits are realised?

It is vital that new healthcare facilities are provided on site to meet the demands of new residents, workers, visitors/tourists, and passengers, especially as some local services are overstretched at present and increased demand could compromise the volume and quality of services provided to already existing communities.

The Estates and Facilities Team for Camden and Islington Primary Care Trusts will be submitting a detailed proposal for healthcare facilities on the King's Cross Central sit as part of the section 106 negotiations between Argent St George and London Borough of Camden, and London Borough of Islington.

The location of any healthcare facilities on the King's Cross Central site is also an important factor to take into consideration. It is vital that any primary healthcare facilities are accessible to all potential users, and are situated in a predominantly residential area (or group of development zones) on a main route. For the walk-in centre, the best location would be in the southern part of the site near to the railway termini and London Underground access points, and also on a main route with suitable access for emergency vehicles.

3.3.5 Key issues to be addressed

In the Environmental Statement, Argent St George raise the possibility that a walk-in centre could be provided by a private healthcare company rather than by the public sector.

Confirmation is needed from Argent St George about the redevelopment of the Triangle site (see Section 1.6 in Part III of this Response). If the Triangle site is not to be redeveloped, there will be the loss of a potential location for a "medi-centre" (as described by Argent St George. If the Triangle site is not developed in which development zone do Argent St George intend to build a primary healthcare facility?

3.4 Access to affordable nutritious food

3.4.1 What will the King's Cross Central redevelopment deliver?

Argent St George have identified a supermarket as a possible use of floor space in 3 development zones (Zones L, P/S, and W).

According to the analysis undertaken by the KX HIA Team, if a supermarket was to be built at King's Cross Central in one or more of three zones – Zones L, W, and P/S – it would be scheduled for completion in 2012-2013, 2014-2015, or 2017-2019, respectively.

Box 3.4 ***Access to affordable nutritious food***

What does the local information base show?

- King's Cross, and St Pancras and Somers Town wards are among the wards with the least access to healthy affordable food of all the wards in Camden and Islington (Rowlinson, 2000)
- Community organisations are concerned about poor access to good-quality food in some areas of Camden, particularly through the supermarket chains, which are cheaper than local stores (Public Health Report, 2000)
- There is a lack of major stores in the King's Cross area (King's Cross Household Study, 2000)
- There is a scarcity of local outlets for affordable healthy food (Di Janus, 2000)

3.4.2 Who will benefit and how?

Any residents in the area would benefit from a shopping facility that sells affordable, nutritious and good-quality food. Affordable food increases the amount of disposable income available for other important uses, e.g. fuel, clothing, transport, access to which will improve health. Eating nutritious affordable food can help towards reducing conditions such as coronary heart disease, cancer, and diet-induced diabetes.

3.4.3 What will be the effect on the inequalities gap?

If a shopping facility is built at King's Cross Central that offers affordable nutritious and good-quality food, it should decrease the inequalities gap, because while benefiting people in the existing and new residential communities it will be particularly beneficial for those whose diet might be poor due to low levels of income,

e.g. older people, unemployed people and their families, lone parents and their families, students, and people who have a chronic illness.

3.4.4 How can we ensure the benefits are realised?

We suggest that:

- at least one of the options for a supermarket at King's Cross Central is realised, and Argent St George negotiate with a chain that produces good-quality but affordable food (e.g. Asda, Costcutter, Lidl);
- whichever supermarket chain is recruited to the site, explore whether they are prepared to be involved in Healthy Eating Campaigns targeted at the vulnerable in the community within the framework of the PCTs' Healthy Eating Policies and Implementation Plans, especially given the rates of coronary heart disease, and cancers in the area;
- the shopping facility is placed in an accessible location on site.

3.4.5 Key issues to be addressed

It is important to ensure the affordability of at least one shopping facility or supermarket in view of the low level of income (<£200 a week) for people living in the communities around the site.

Confirmation is needed from Argent St George about the redevelopment of the Triangle site (see Section 1.6 in Part III of this Response). If the Triangle site is not to be redeveloped there will be the loss of a potential location for a relatively small supermarket. Such a loss may reduce the capacity of King's Cross Central to generate health improvement through improving the accessibility of affordable good-quality nutritious food.

3.5 Access to education, community and cultural facilities

3.5.1 What will the King's Cross Central redevelopment deliver?

From the information Argent St George have provided, it is not possible to identify the specific community and cultural uses that will be delivered in each development zone and at which time-point. However, Argent St George have made provision for D1 land uses, which includes community and cultural uses, in every group of development zones scheduled for completion at the same time.

The types of facilities that Argent St George could provide are library facilities, community centre facilities, youth facilities, day care facilities, day nursery facilities, primary school provision, higher education colleges, a visitor/tourist information centre, industrial heritage and other museums, art galleries/visual arts centre/exhibition space, and enhanced facilities for boat users.

Argent St George have stated that there are currently in discussion with a higher education institution to occupy 40,000 square metres in development Zone L, scheduled for completion in 2013-2014.

From the analysis completed by the KX HIA Team, D1 land uses could be delivered at the following time-points: 2012-2013, 2013-2014, 2014-2015, 2015-2016, and 2017-2019, which coincide with the completion of zones containing residential units.

Box 3.5 Access to education, community and cultural facilities

What does the evidence base show?

- Community centres are part of the therapeutic landscape, quasi-public places where people meet, greet and nurture social relations (Witten et al, 2001)
- In the absence of safe places that offer opportunities for social interaction, people's sense of belonging to a particular community and place is diminished (Warin et al, 2000)
- In a Swedish study of urban neighbourhoods, people in an area that had undergone improvements to local services and facilities had reduced levels of mental illness and increased levels of social support (Dalgard and Tambs, 1997).
- Community facilities provide venues for health-promoting activities, and informal meeting places outside the home and work where social relationships can be formed and maintained (Witten et al, 2001).
- The provision of community facilities, however, does not constitute or develop social capital - it is only a material base on which social capital can be built; for social capital to develop, it requires greater participation and representation of the community in the growth and development of facilities, which helps to meet constantly changing social conditions, and a range of needs (Campbell et al, 1999).

What does the local information base show?

- In an audit of community centres in Camden, it was stated that community buildings could have a vital role in the regeneration process, contributing to the strengthening of communities by combating social isolation and encouraging social cohesion.

The only zone in which Argent St George mentions a dedicated community facility is Zone W, the Triangle Site scheduled for completion in 2014-2015 (see page 12-64, paragraph 12.7.148 of Environmental Statement). Otherwise, leisure uses (in the broad sense of the term) are treated or interpreted as "communal spaces".

3.5.2 Who will benefit and how?

Any residents in the area, whether new or existing, could benefit from community facilities through increased social contact, networking and social support as well as having an increased sense of belonging to the area and the community. There will also be benefit from taking part in stress-relieving activities, whether community-based or cultural. In zones where there is mixed tenure, community facilities could provide a foundation upon which social integration and following that community cohesion could be achieved.

Workers seeking rest and recreation after work will benefit also from the close proximity of cultural facilities, and the capacity to take part in stress-relieving activities.

3.5.3 What will be the effect on the inequalities gap?

If community facilities are well represented in the redevelopment, they could serve to reduce the inequalities gap, by increasing the potential for social integration and inclusiveness, not only between people in market housing and people in affordable housing on site, but also between the new community of the King's Cross Central site and the existing local communities around the site. However, if community facilities are under-represented, it is possible that this could exacerbate inequalities by

increasing the level of social exclusion, by not providing the "substrate" on which social integration could take place.

3.5.4 How can we ensure the benefits are realised?

We strongly suggest:

- that community facilities are provided on site in tandem with the phasing of the construction of residential units;

We suggest that:

- smaller neighbourhood meeting places or centres could be built in the some of the development zones containing residential units;
- new residents on site be given the opportunity to participate in the growth and development of any community facilities;
- consideration is given to the need for facilities that can be used as faith facilities, especially in view of the lack of such facilities for the Muslim communities in the area;
- the needs of children, young people, and older people are taken into account when planning community facilities;
- the cultural needs of the various black and minority ethnic groups in the area are taken into account when planning community facilities.

3.5.5 Key questions to be addressed

If negotiations with the Higher Education Institution for 40,000 square metres in Zone L are not successful, will this floor space be allocated to another category of land use?

Are Argent St George planning to build only one dedicated community centre on the Triangle site, and have none on the Main site? This is especially important in view of the uncertainty that appears to be associated with the redevelopment of the Triangle site (see below).

Confirmation is needed from Argent St George about the redevelopment of the Triangle site (see Section 1.6 in Part III of this Response). If the Triangle site is not to be redeveloped there will be a loss of a dedicated community space/facilities and childcare facilities. Such a loss would reduce the capacity of King's Cross Central to generate health improvement, especially for new and existing residents in the area.

3.6 Access to leisure facilities

3.6.1 What will the King's Cross Central redevelopment deliver?

From the information Argent St George have provided, it is possible to identify only a few of the specific assembly and leisure uses that will be provided at specific time-points. However, Argent St George have made provision for D2 land uses, which includes assembly and leisure, in every group of development zones scheduled for completion at the same time-point.

D2 category uses include concert halls, dance halls, nightclubs, casinos, gymnasiums, sports centres, swimming pools and other sports/recreation areas, including cinemas.

From the analysis undertaken by the KX HIA Team, the time-points at which completed zones containing assembly and leisure uses will be delivered are 2012-2013, 2013-2014, 2014-2015, 2015-2016, and 2017-2019. Thus, the provision of leisure facilities has been scheduled to coincide with the increase in the number of residents, workers, and others visiting the site.

The specific assembly and leisure uses Argent St George have indicated might be built include:

- a leisure centre in Zone W, the Triangle site, with either a sports hall or swimming pool, due for completion in 2014-2015 (cited in the Triangle Explanatory Statement, and the Regeneration Strategy);
- a range of health clubs and other indoor sport facilities, ice-skating opportunities (in a multi-functional public realm), and boule, tennis, wall tennis and areas for 5-aside football (cited in the Regeneration Strategy);
- a cinema in any one of several zones in the north of the site – Zones L, R, and P/S – delivery dates for these completed zones are 2013-2014, 2013-2014, and 2017-2019, respectively.
- specialist play areas for 1-8 year olds, including one in Zone N (due for completion in 2012-2013);
- places for 8 -16 year olds to play;
- a rock-climbing wall or more challenging activities in Zone N (due for completion in 2012-2013).

Box 3.6 ***Access to leisure facilities***

What does the evidence base show?

- Leisure is essential to an individual's psychological health (London Health Commission, 2002)
- "Leisure lack" has been equated with a diminished sense of well-being (Neulinger, 1982)
- Children's play territory has been reduced as roads and pavements have become more dangerous (Hillman et al, 1990).
- There has been a reduction in the opportunities for children to embark upon independent play and exploration due to the fear of crime (Valentine and McKendrick, 1997).
- The provision of play facilities that allow expression of independence and play mobility may have positive behavioural and mental health benefits for children (London Health Commission, 2002)
- Physical exercise can be beneficial in the prevention and treatment of some of the most common causes of physical ill health including coronary heart disease (CHD), hypertension, obesity, osteoporosis, diabetes mellitus, and asthma (Thomas et al, 1981).
- The incidence of coronary heart disease in middle-aged men is approximately halved in the most active groups when compared with the most sedentary (Berlin and Colditz, 1990).
- Being physically active reduces the risk of later hip fracture by 50% (Joakimsen et al, 1997).
- Physical activity may also play an important role in the management of common mild-to-moderate mental health diseases, especially depression and anxiety (Thomas et al, 1981; Paluska and Schwenk, 2000).
- Regular participation in an exercise programme has been found to be associated with measurable increases in self-esteem in adults and children (Macauley, 1994).
- Regular exercise may reduce memory loss in older people (Thomas et al, 1994).
- Strenuous physical activity improves perceived health, and it reduces the risk of heart disease, stroke, hypertension, atherosclerosis, colon cancer, lower back pain, osteoporosis, and adult-onset diabetes (Berkman and Breslow, 1983).
- The provision of a variety of opportunities, acceptable to people of all ages, abilities and backgrounds, to engage in appropriate physical activity can have a direct and positive impact on population health (London Health Commission, 2002)

What does the local information base show?

- In the "Mapping Mildmay" Project, participants identified the need for more open access play areas (Healthy Islington)
- There are few affordable, structured activities to enhance children's quality of life and health. Existing leisure facilities are seen as expensive and outside the reach of many local families (Di Janus, 2000)
- A common theme in health needs assessment in the area is the lack of facilities, including those for sports and leisure

3.6.2 Who will benefit and how?

There is the potential for any resident to benefit from assembly and leisure facilities, through stress-relieving activities and increased social contact, networking and support. However, the capacity for the vulnerable on site and in the surrounding community to benefit from such facilities depends on two main factors: affordability, and whether the facilities provide for activities that are of interest to them.

Workers seeking rest and recreation after office hours would benefit from assembly and leisure uses through stress-relieving activities. Similarly, for visitors to the site, there will also be this benefit.

3.6.3 What will be the effect on the inequalities gap?

It is possible that the leisure and assembly uses on site could help to reduce the inequalities gap if there are enough facilities that are open access, affordable and of interest to the vulnerable groups and communities in the surrounding area. If affordable facilities, or those that have open access, are not provided, it is likely that the inequalities gap will widen.

3.6.4 How can we ensure the benefits are realised?

We suggest:

- that the balance of assembly and leisure uses on site is built to be socially inclusive, such that all residents in the area, whether new or existing, could access them;
- that, where relevant, leisure facilities, but especially sports facilities, have the capacity to provide culturally sensitive services, e.g. swimming pool that is managed in such a way as to accommodate the needs of the Muslim communities in the area;
- that the entrance/usage cost of leisure facilities, including sports facilities, are affordable, especially in view of the number of people living on a low income in the area surrounding the site;
- the needs of the relatively young resident population surrounding the site are considered when deciding on the balance of leisure and assembly uses.
- that the estate management strategy/protocol takes into consideration the potential impacts of a 24-hour economy on residents and other users, and incorporates a mechanism to liase with the licensing department at the Council.

We endorse:

- the inclusion of a leisure centre with sports facilities in Zone W, although it is important to ensure that these facilities are affordable;
- the inclusion of a specialist play area in Zone N, and other specialist play areas, for 1-8 year olds;
- the inclusion of places for 8-16 year olds to play;
- the inclusion of areas/facilities for 5-a-side football.

3.6.5 Key issues to be addressed

- Affordability of leisure facilities at King's Cross Central
- Social inclusiveness of leisure facilities at King's Cross Central
- Cultural inclusiveness of facilities at King's Cross Central
- Potential for development of a 24-hour economy at King's Cross, and the nuisance this could cause to residents on site, other users of King's Cross Central, and residents in the surrounding areas.
- Confirmation is needed from Argent St George about the redevelopment of the Triangle site (see Section 1.6 in Part III of this Response). If the Triangle site is not to be redeveloped, there will be a loss of a leisure facility, which could be

either a sports hall or a swimming pool. Such a loss would reduce the capacity of King's Cross Central to generate health improvement, particularly for existing residents.

3.7 Access to transport

3.7.1 What will the King's Cross Central redevelopment deliver?

Argent St George are developing proposals with both London Buses and Transport for London to introduce bus services to King's Cross Central, and to enhance and improve existing bus services through and around the site.

Box 3.7 ***Access to transport***

What does the evidence base show?

- Transport promotes health through the achievement and maintenance of social networks (Davis, 1998)
- As traffic volumes increase, the density of friendships and people's sense of neighbourliness decreases (Appleyard, 1981).
- Disadvantaged urban areas tend to be characterised by high traffic volume, leading to increased levels of road traffic accidents, whilst lacking the benefits of access to public transport (Davis, 1998).
- The risk of injury (Roberts et al, 1998), especially for child pedestrians, increases with:
  - traffic volume;
  - a high density of curb parking;
  - mean traffic speeds over 40 kph.
- Children's play territory has been reduced as roads and pavements have become more dangerous (Hillman et al, 1990), which will have a further effect on health by curtailing the child's sense of independence and personal mobility, and thereby affecting psychological development (Pain, 2001).
- Road traffic accidents have a social class gradient: in the UK, deaths for children in the poorest families (social class V) are more than four times greater than those in the richest (social class I) (Jarvis et al, 1995). This inequality will persist as long as the safety of vehicles depends on a willingness to pay for safety features when purchasing a car (Roberts and Power, 1996).

Argent St George have planned the redevelopment such that it encourages active transport, especially with regard to ensuring safety. In certain areas of the site, pedestrians and cyclists are segregated from other traffic, however, the overall aim for the site is developing shared spaces for the different transport modes

Argent St George have also developed a Green Transport Plan.

3.7.2 Who will benefit and how?

Any residents in the area will benefit from the increased accessibility to public transport and active transport (walking and cycling) options, transport being the determinant of health that facilitates access to and availability of other determinants of health, such as employment opportunities, various services, facilities, and amenities, and social contact, networking and support. In addition, the inclusion of active transport options in the redevelopment may induce some residents, workers and

visitors to the site to increase the amount of exercise taken as part of their daily lives, thereby increasing cardiovascular and muscular fitness and reducing weight and obesity, with the potential to help reduce the incidence of heart disease.

All users of the site will benefit from the implementation of the Green Travel Plan if it reduces the amount of private car use, to, from and through the site. There would be a reduction in the potential number of traffic accidents and a reduction in emissions to air, particularly as the site falls into an Air Quality Management Area (AQMA). A reduced number of road traffic accidents could be particularly important for children both on site and in the surrounding area, and the reduced emissions to air could be beneficial to the young, the old, and those already suffering from respiratory and cardiac conditions.

3.7.3 What will be the effect on the inequalities gap?

It is possible that increasing accessibility to public transport could reduce the inequalities gap by enabling the vulnerable in the local community to have greater access to various opportunities, services and facilities that might improve their health.

The effects of implementing the Green Travel Plan, i.e. reducing road traffic accidents and potentially improving air quality, might be expected to have greater positive impacts for the vulnerable and thereby reduce the inequalities gap. This is because the vulnerable are more likely to experience differential negative impacts, i.e. more likely to be involved in a road traffic accident, and more likely to have a pre-existing respiratory or cardiac condition when compared with a healthy population.

3.7.4 How can we ensure the benefits are realised?

We endorse:

- the intention to introduce bus services to the King's Cross Central site, and to improve and enhance existing services;
- the plans to make active transport options viable alternatives to the use of non-active transport;
- the principle of developing and implementing a Green Travel Plan for the King's Cross Central site, and the decision to nominate a Travel Coordinator to work with occupiers and other stakeholders to deliver on the objectives.

We suggest:

- that the nominated Travel Coordinator liaise with healthcare providers in the area, particularly the primary care trusts, to ensure that any health-related initiatives to increase daily physical activity in relation to transport are coordinated, and opportunities for joint working are identified and capitalised upon.

3.7.5 Key issues to be addressed

It would appear that the impacts of traffic on existing local communities has not been a focus for the Transport Impact Assessment. We are concerned to see further investigation of the impact parking by non-residents may have, the impact private cars and taxis may have when dropping off at the railway termini, and also the impact of the potential short-cuts people may take using local roads, for instance, Eversholt Street, Brill Place, and Purchese Street.

3.8 Access to green and open spaces

3.8.1 What will the King's Cross Central redevelopment deliver?

The only green space included in the redevelopment of King's Cross Central is the Long Park in the north of the site.

However, there are planting schemes throughout the site.

Box 3.8 ***Access to green and open spaces***
What does the evidence base show?

- Access to open spaces can increase the level of exercise undertaken in a community, thereby contributing to reducing obesity, cardiovascular disease, diabetes, and arthritis. The impact on levels of exercise is most likely to be experienced by children (Michie and de Rozarieux, 2001).
- Access to green spaces can improve social contact and interaction, and increase community activities, thereby contributing to reducing stress-related problems (Michie and de Rozarieux, 2001).
- Access to parks has a positive influence on measures of children's behaviour, including the number and nature of friendships, and characteristics of play patterns (McCulloch and Joshi, 2001; Parke and Bhavnagri, 1989)
- Criminal, social or psychological aggression often takes place in green spaces (Michie and de Rozarieux, 2001).
- Drug abuse and conduct offences may be centred in green spaces (Michie and de Rozarieux, 2001).

3.8.2 Who will benefit and how?

Any residents, workers, visitors or passengers in the area would benefit from access to green and open spaces, allowing them space for exercise, physical activity, play, and social contact and interaction.

The location of the Long Park may increase access to green and open spaces for vulnerable people, which means that any benefits could be shared more equally throughout the community.

3.8.3 What will be the effect on the inequalities gap?

There is a possibility that the provision of accessible green and open space at King's Cross Central could help to reduce the inequalities gap, by encouraging greater use in the vulnerable groups.

3.8.4 How can we ensure the benefits are realised?

We endorse:

- the inclusion of the "Long Park" in the north of the King's Cross Central site;
- the inclusion of planting schemes throughout the site as illustrated in the Landscape proposals.

We suggest:

- that the Estate Management strategy/protocol covers the Long Park appropriately in that green spaces can be a location for drug abuse and conduct offences, including various types of aggression.

We ask Argent St George to consider:

- identifying further possibilities to introduce green space into the King's Cross Central redevelopment.

3.9 Quality of landscape/Level of biodiversity

3.9.1 What will the King's Cross Central redevelopment deliver?

Argent St George intend to create a wildlife habitat on the Triangle site, with a long-term view in particular to encouraging the establishment of breeding pairs of black redstarts.

3.9.2 Who will benefit and how?

Residents in the immediate vicinity of the wildlife habitat will experience the greatest benefit, either through being able to see it or through being able to see some of the wildlife that live in or visit the site (e.g. birds and butterflies visiting vegetation in their vicinity or in other parts of the site). The potential impacts on health will be an increase in mental well-being through the relief of stress and anxiety.

3.9.3 What will be the effect on the inequalities gap?

It is highly unlikely that the changes to the landscape at King's Cross Central will affect the inequalities gap.

3.9.4 How can we ensure the benefits are realised?

We endorse Argent St George's development of a wildlife habitat on the Triangle site, especially as it is designed to house black redstarts, a rare breeding bird in the UK. This would increase the overall amount of wildlife habitat in the area, and owing to its relative proximity to Camley Street Natural Park may act to support the existing wildlife populations in the area.

3.10 Quality of townscape

3.10.1 What will the King's Cross Central redevelopment deliver?

Argent St George have undertaken to develop a new quarter for London, including new streets, offices, housing, shops, food and drink outlets, a park, and leisure and entertainment facilities. They also propose to develop the public realm throughout King's Cross Central by building new squares and open spaces including attractive features such as sculpture and fountains. Their vision for the townscape is contained in the Urban Design Statement submitted as one of the supporting documents.

Box 3.10 ***Quality of townscape***

What does the evidence base show?

- Good design encourages greater ownership, and involvement of communities, and can reduce negative effects such as vandalism, and the under-use of facilities (Evans and Shaw, 2001; Wilson, 1987).
- After a series of environmental improvements to an English town estate (Halpern, 1995), the following changes were recorded in residents:
  - reduced anxiety and depression;
  - improved self-esteem;
  - reduced fear of crime;
  - a perception of greater friendliness of the area.

3.10.2 Who will benefit and how?

It is likely that everyone, including existing communities, new residents, workers, and visitors, will benefit from an improvement to the quality of townscape represented by Argent St George's plans for King's Cross Central. In terms of health, these benefits will be apparent as improvements in mental well-being, and an increase in social contact, and networking, which will also have a positive effect mentally.

However, it is possible that some residents in existing communities, especially those living in some of the surrounding estates, could feel resentment at the difference between the quality of townscape (environment) at King's Cross Central when compared with that where they live. This resentment could have a negative impact on mental well-being, and may exacerbate any pre-existing mental health problems

3.10.3 What will be the effect on the inequalities gap?

It is not certain how the changes to the townscape will affect the inequalities gap. There are several possible outcomes, as follows:

- the inequalities gap remains the same because the health and well-being of everyone increases to the same degree;
- the inequalities gap decreases as people in the existing communities suffering from health inequalities gain greater benefit from the new townscape than those who are already healthy and accustomed to living in a good-quality townscape/environment;
- the inequalities gap widens for those people in existing communities who feel resentment, and therefore a negative impact on mental well-being, with respect to the quality of their environment when compared with that in the new quarter of King's Cross Central;
- the inequalities gap widens to a greater degree as people who are already healthy experience health gain as a result of the new townscape and the health and well-being of those who are already suffering from health inequalities deteriorates due to feelings of resentment.

3.10.4 How can we ensure the benefits are realised?

We welcome:

- Argent St George's intention behind the preparation of the Urban Design Statement.

We endorse:

- Argent St George's consideration of the potential to incorporate green and brown roofs on the Main site and the Triangle site, which will also have a positive effect on microclimate at King's Cross Central.

We ask:

- Argent St George to consider effective and appropriate ways of promoting social inclusion within King's Cross Central, including the design of good-quality open-access facilities

3.11 Crime and community safety

3.11.1 What will the King's Cross Central redevelopment deliver?

Argent St George intend to deliver a new public realm for the redevelopment of King's Cross Central, and to incorporate crime prevention design principles into the design of the redevelopment.

They have also endeavoured to generate "active" street frontages throughout the King's Cross Central site.

Argent St George plan to set up an Estate Management Company responsible for some community safety issues. This is similar to a scheme Argent St George have implemented at Brindleyplace in Birmingham.

Box 3.11 ***Crime and community safety***

What does the evidence base show?

- Linking community safety entirely with the design of the built environment shifts the focus away from other causes of crime that are social and political (Koskela and Koskela, 2000).
- Communities characterised by anonymity, and limited acquaintances among residents, unsupervised teenage peer groups, and low levels of civic participation face increased violence, and crime (Putnam, 2000).
- Residential mobility is associated with high levels of crime and victimisation (Crutchfield et al, 1982).
- Poverty contributes to criminality only in transient communities characterised by rapid population turnover (Sampson, 1985).
- Social capital is important for discouraging antisocial or criminal behaviour (OECD, 2001).
- Violent crimes tend to cluster around licensed premises (Mirlees-Black et al, 1998; Speer et al, 1998), and peak at weekends and between 2300 and 0200 (Mirlees-Black et al, 1998; Deeham, 1999).
- In the 1995 British Crime Survey, it was indicated that in 53% of violent crimes by strangers and in 45% of violent crimes by acquaintances the offender was under the influence of alcohol. Alcohol Concern (1991) estimated that the offender was under the influence of alcohol in 78% of violent crimes. Bromley and Nelson (2002) found alcohol to be a contributory factor in:
  - all recorded crimes (8% of total);
  - harassment crimes (48% of total);
  - violent crime (36% of total);
  - criminal damage (16% of total).
- Bromley and Nelson (2002) also found that most alcohol-related crimes occur in the city centre at night. Major "hot spots" of alcohol-related crime are:
  - pubs and clubs;
  - confluence flash-points and route-ways where people are channelled by the built environment.

  At around midnight, alcohol-related crime and disorder is clustered around pubs and clubs. After midnight, the incidence of crime and disorder follows people moving away from the city centre's night-time leisure zones along key route-ways to residential areas. Criminal damage occurs along these route-ways. Scope may exist for planning out alcohol-related crime and disorder by avoiding clusters of functional activities that are clearly associated with the problem.
- Improvements to street lighting reduce crime (Pease, 1999) through both precisely targeted increases, and, in the UK, general increases.
- In the most recent and sophisticated studies, improvements to street lighting have been associated with reductions in crime during the day as well as during the hours of darkness (Pease, 1999).
- CCTV can be effective in deterring property crime, but the findings are mixed when looking at its effect on personal crime, and public order offences (Phillips, 1999).
- A drop in crime does not always result in a drop in fear of crime (Crime Concern, 1999) – a sustained reduction in local levels of crime may have a more direct effect than a reduction in crime figures nationally.
- In general, untargeted increases in crime prevention make residents feel less fearful of crime or more confident of their own safety at night (Pease, 1993).

- Although before installation, people thought the presence of CCTV cameras would make them feel safer, after installation there was no improvement in their fear of crime (Ditton, 2000). People stated that police patrolling is more effective than CCTV in making them feel safer.
- Designing out the fear of crime is based on an assumption that most crime is opportunistic, and offenders respond in a mechanistic way to environmental stimuli (Walklate, 1995).
- In some cases, the causes of crime may be related to health - there is a strong link between drug misuse, unemployment, truancy, and other aspects of social exclusion and increased levels of antisocial and criminal behaviour including domestic violence, assault, and fires. It is thought that up to three-quarters of certain crimes are drug-related, and up to two-thirds of burglaries and violent crime may be committed under the influence of alcohol (Robinson et al, 1998).

What does the local information base show?

- In the 1996 Health Needs Assessment, it was found that although policing could be effective in reducing drug-related activities and crime, and prostitution, there tended to be a spatial displacement effect which just shifted activity to another part of King's Cross.
- In the 2002 Health Needs Assesment, the King's Cross area had twice Camden's average crime rate for domestic violence, drugs crimes, racial harassment and alcohol-related crime. The police cited drug-related problems, antisocial behaviour, racial harassment and violence as the most common problems in King's Cross.
- The streets of King's Cross are generally viewed as unsafe, leading parents and carers to keep young children inside to play (Di Janus, 2000).

3.11.2 Who will benefit and how?

All users of the site would benefit to a certain extent from a design and build of King's Cross Central that reduced the opportunities for crime and antisocial behaviour on site.

However, as King's Cross has been a hotspot for crime for many years, if crime is reduced on site, it is likely that it will be displaced, and dispersed elsewhere. This has two implications:

- people living in areas nearby will experience the effects of the displaced crime and antisocial behaviour;
- it will be more difficult to combat crime and antisocial behaviour if it is dispersed over a wider area.

For instance, the construction of Channel Tunnel Rail Link (CTRL) displaced sex-working and drug-taking and drug-dealing activities to as far north as the Market Road-Caledonian Park-Market Estate area.

However, it is possible that the nature of the redevelopment, in conjunction with the major transport hub/port at the south of the site, may generate different types of crime and antisocial behaviour. In particular, we would anticipate an increase in alcohol-related crime (e.g. violence, harassment, and criminal damage), noise nuisance and antisocial behaviour as a result of the increased provision of licensed premises in the area.

3.11.3 What will be the effect on the inequalities gap?

The inequalities gap in the wider area/across the borough could increase if the pattern of crime and antisocial behaviour traditionally seen in the King's Cross area is pushed out into the surrounding estates, and people may be subject to crime and antisocial behaviour which they had not experienced before, or at a greater level than they had experienced before. In contrast, it is likely King's Cross Central itself may have relatively low levels of crime traditional to the area. However, it is possible that the nature of crime at King's Cross could change (e.g. an increase in pickpockets) and its proximity to the transport hub/port may also have an impact (e.g. smuggling of goods).

3.11.4 How can we ensure the benefits are realised?

We support:

- the use of crime prevention design principles (secure by design) in the design of the King's Cross Central redevelopment;
- the introduction of an Estate Management Team to help deal with community safety issues not only on the King's Cross Central site but also around the margins of the site into the existing local communities – however, it is vital to address the issue of displacement of crime and antisocial behaviour, and what this could mean for other communities and service providers such as the police, and the street warden service.

3.11.5 Key issues to be addressed

- The displacement of crime and antisocial behaviour from the immediate vicinity of King's Cross to other parts of both boroughs, which are likely to be areas where vulnerable people live
- The possibility that the crime and antisocial behaviour profile that has traditionally been seen in the King's Cross area changes as a result of the operation of the Port to include new crimes/patterns of antisocial behaviour, for instance, increase in arms smuggling, increase in drugs smuggling, increase in pirate merchandise, increase in risk of terrorist attack.
- The need to ensure that the concentration of office space in the south of the site next to the railway termini does not create a "no-go" area at night due to the lack of residential floor space.

3.12 Public safety: incidents or accidents at or affecting King's Cross Central

We note the change in the layout of the southern part of the site (from that presented in the Third Consultation document), and acknowledge the efforts Argent St George have made to prevent the main north-south route from becoming a funnel, especially in the event of an emergency and having to evacuate either or both of the railway termini.

3.12.1 How can any harms be minimized?

We ask that:

- Argent St George and landowners take responsibility for their contribution to Emergency Planning and Port Health as laid out in paragraph 2.10.10 of Joint Planning and Development Brief.

3.12.2 Key questions to be addressed

- Given that this is a high-density mixed use development, it is important that all occupiers have suitable arrangements in place in the event of an emergency, and need to evacuate buildings. Will coordinating these arrangements form part of the Estate Management Strategy for the site? There is a need for co-ordination on this issue to ensure that the potentially large number of people on site can be managed quickly and effectively, especially if the railway termini and/or Underground need to be evacuated at the same time.

3.13 Construction Impacts/Code of Construction Practice

3.13.1 Construction impacts

Argent St George have identified the need to undertake some construction at night - augured and percussive piling (page 13-53 paragraphs 13.7.85 and 13.7.86 respctively).

- We ask that any 24-hour working is kept to an absolute minimum, especially in view of the findings of the Planning Inquiry into 24-hour working for the Channel Tunnel Rail Link (CTRL) Project.

3.13.2 Code of Construction Practice (COCP)

Policy/Strategy drivers

London Borough of Camden's Race Equality Scheme (RES) aims to reduce and ultimately remove barriers so that black and minority ethnic communities and those with certain religious beliefs have equal rights and opportunities with others in the borough.

We ask that:

- the communities/residents identified by Arup to be most affected by King's Cross Central construction works (people in Rufford Street, Gifford Street, York Way, new housing on corner of Rufford and Gifford Streets, and houseboats on Regent's Canal) are met with every two weeks and consulted about the works in the same way as residents in Coopers Lane and Goldington Estates following the Planning Inquiry into 24-hour working on the CTRL Project.
- Argent St George identify more effective ways of communicating with the local communities, particularly those from black and minority ethnic groups, than those identified in the COCP, which lists a focal point for information dissemination via a helpline, website, information boards and exhibitions.

It should be taken into account that:

- helplines are not widely used by local communities, as evidenced by the Planning Inquiry into 24-hour working on the CTRL Project;

- with respect to a web site, not everyone has access to information technology (IT) – half of Camden residents do not have access to a computer either at home or at work[8].

It is more effective to disseminate information for the community through community locations, such as schools, GP surgeries, community centres, local shops, than have information at one point of dissemination.

In addition, many of the people in the surrounding communities are from black and minority ethnic groups, particularly Bangladeshi, Somali and Turkish speakers. To our knowledge, Argent St George have only ever produced consultation documents in the English language. From the profile in Box 2.2 of this document, it is clear that there are many people in the local community who have no or low English language skills. It is vital that information, even if it is only summaries, is produced in the main languages of the black and minority ethnic groups in the area. Otherwise these vulnerable groups are automatically prevented from accessing information about the redevelopment, its construction, and the ways in which they could respond. This has been a consistent finding during the Community Workers in HIA Project, run as part of the KX HIA but led by a combination of voluntary and public sector organisations in Camden and Islington.

We also ask that:

- Argent St George consider including a behavioural Code of Conduct for nominated contractors. During the Community Workers in HIA Project, girls and young women in the local community have reported being subject to inappropriate comments by CTRL construction workers, which they have found offensive and intimidating.

3.13.2.1 Part D – Monitoring

D2.2 – We ask that Argent St George consider setting up regular noise monitoring – at present the COCP merely states "from time to time during construction", which gives the impression of being random and occasional. There is no mention in this section of Argent St George instituting a mechanism to investigate and deal with noise complaints, and yet there is such a process for dust under Air Quality Monitoring.

[8] Data from London Borough of Camden's Community Strategy and Neighbourhood Renewal Strategy Baseline Evaluation 2002

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